

ENGLISH FOR THE DENTAL TEAM

Dental Assisting English

Clarify the request. Support the patient.

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Eight lessons. Eleven extended conversations.

INSIDE THE BOOK

192 vocabulary entries / 128 reusable phrases
220 extended-dialogue turns / 11 transfer exchanges
Word banks and explained answer keys

Upper-intermediate to advanced (B2-C1). For dental assistants, dental nurses, chairside support staff, and dental practice teams.

2026 edition

Eight scenarios. Eight skills.

Clarify appointments and chairside requests, support patient questions, check records, track laboratory work, and communicate aftercare concerns. Three further cases cover tooth notation, medical history, and dental-material stock.

01 Clarifying the appointment purpose

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Clear language supports careful teamwork.

Dental assisting combines technical vocabulary, short chairside exchanges, and patient conversations that need time and empathy. A clear question can prevent a wrong assumption. A helpful explanation can acknowledge uncertainty without sounding dismissive.

Separate the appointment from the treatment

A schedule entry, a patient expectation, and a clinical treatment decision are different pieces of information. Explain the recorded visit and arrange clarification before assuming agreement to a procedure.

"The schedule shows a consultation; the dentist needs to clarify the plan with you."

Make a concern specific

Name the item, missing check, and action needed. Keep an unverified status distinct from a confirmed failure, but do not let schedule pressure turn missing evidence into permission to proceed.

"I cannot confirm the readiness record for this pack; we need the required check."

Check the word you missed

Chairside speech can be brief and interrupted. Ask for the distinguishing word and repeat the full request. Knowing two possible instruments does not tell you which one was requested.

"Did you mean the instrument tray or the impression tray?"

Route clinical questions without abandoning them

An assistant can recognize and communicate a question while staying within the actual role. Preserve the patient concern and identify the clinician or approved process that can answer it.

"I cannot interpret the image, but I will ask the dentist to explain what it shows."

PROFESSIONAL BOUNDARIES

All patients, staff, records, schedules, and cases are fictional. This book teaches workplace English, not dental treatment, radiographic interpretation, infection-control certification, or a universal scope of practice. Follow current local law, authorized duties, training, manufacturer instructions, and practice procedures. Use the appropriate urgent or emergency route for urgent concerns; routine callback examples do not replace it. United States and UK sources provide context, not interchangeable legal rules.

A consultation is not a confirmed filling

A Read the situation

Reconcile a patient expectation with the schedule and arrange an explanation before assuming a treatment decision.

Patient Lena arrives expecting a filling today. Assistant Amir sees a thirty-minute consultation on the schedule; no procedure is confirmed. The dentist is available to clarify the purpose before Lena decides whether to continue. Amir should acknowledge the time she has set aside, explain what the record actually shows, and preserve her questions about options, costs, and timing. He cannot promise a filling today, diagnose the tooth, or imply that attending a consultation means agreeing to whatever treatment might later be recommended.

PEOPLE / Lena: Patient | Amir: Dental assistant

CORRECT THE EXPECTATION WITHOUT BLAMING THE PATIENT

A mismatch may result from several conversations or records. Explain what you can verify and invite clarification without declaring that the patient heard incorrectly. A neutral correction protects trust while leaving clinical decisions with the appropriate person.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What does the schedule show?

- A A thirty-minute consultation
- C A guaranteed procedure today

- B A completed filling
- D An emergency extraction

2. What does Lena expect?

- A A laboratory delivery next week
- C No discussion with a dentist

- B Only a telephone call
- D A filling today

3. Who can clarify the clinical plan?

- A The schedule automatically
- C Amir by guessing from the booking length

- B The dentist before Lena decides whether to continue
- D Another patient in the waiting room

Answers and explanations: [page 85](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

consultation

An appointment for discussion and assessment, not automatic agreement to treatment.

IN USE attend a consultation

restoration

Repair or replacement of damaged tooth structure using an appropriate dental method.

IN USE discuss a restoration

filling

Material placed to restore a prepared tooth area as clinically indicated.

IN USE schedule a filling

appointment type

The category of visit recorded in the schedule.

IN USE verify the appointment type

chief concern

The main issue the patient wants addressed.

IN USE clarify the chief concern

presenting complaint

The problem or symptom reported at the visit.

IN USE record the presenting complaint

clinical examination

Assessment performed by the appropriate clinician.

IN USE arrange a clinical examination

treatment plan

The proposed sequence of dental care discussed with the patient.

IN USE explain the treatment plan

treatment option

A possible approach requiring appropriate explanation and assessment.

IN USE compare treatment options

informed consent

Agreement through the applicable process after relevant understandable information.

IN USE support informed consent

provisional recommendation

A recommendation not yet finalized or confirmed.

IN USE qualify a provisional recommendation

definitive treatment

Treatment intended to address the identified condition rather than an interim measure.

IN USE discuss definitive treatment

Precision check

A thirty-minute booking describes allocated time, not clinical suitability or consent for a filling. Explain the visit type and arrange the clinical discussion. Do not convert a hoped-for treatment into a confirmed procedure because the patient has arrived.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

caries

Tooth decay; identification and management require clinical assessment.

IN USE discuss diagnosed caries

restorative dentistry

Dental care concerned with repairing or restoring teeth and function.

IN USE explain restorative dentistry

procedure code

A coded description of a dental service within a particular system.

IN USE verify the procedure code

treatment estimate

An estimated cost for specified proposed care.

IN USE request a treatment estimate

coverage estimate

An estimate of relevant payment coverage, not a guarantee.

IN USE qualify a coverage estimate

referral

A request for assessment or care by another appropriate provider.

IN USE confirm the referral

review appointment

A follow-up visit to reassess an issue or treatment.

IN USE arrange a review appointment

booking discrepancy

A difference between the recorded booking and the expected visit.

IN USE resolve a booking discrepancy

clinical decision

A decision requiring the appropriate clinical assessment and authority.

IN USE refer a clinical decision

appointment duration

The allocated length of the visit.

IN USE confirm appointment duration

patient expectation

What the patient believes will happen.

IN USE acknowledge a patient expectation

decision time

Time for the person to receive information and make the relevant choice.

IN USE allow decision time

Precision check

The treatment estimate, coverage estimate, and final charge may describe different things. Keep their scope and confirmation status clear. The assistant can help route cost questions without promising insurance payment or diagnosing what treatment will be required.

Phrases you can actually use

C Notice the language / 1 of 2

ACKNOWLEDGE THE EXPECTATION

"I understand you expected a filling today."

STATE THE RECORD

"The schedule shows a thirty-minute consultation."

NAME THE DIFFERENCE

"That differs from the treatment you were expecting."

AVOID BLAME

"Let us clarify the appointment before making assumptions."

INVITE THE CONCERN

"What would you like the dentist to address first?"

EXPLAIN THE NEXT STEP

"The dentist can discuss the purpose and options with you."

KEEP THE PROCEDURE UNCONFIRMED

"No filling is confirmed in the booking I can see."

RESPECT THE DECISION

"You can ask questions before deciding whether to continue."

The schedule shows

Identifies the source and limit of the information.

No filling is confirmed

States current booking status without saying a filling is never possible.

Before deciding

Preserves the patient's opportunity to understand and choose.

Phrases you can actually use

C Notice the language / 2 of 2

AVOID A DIAGNOSIS

"I cannot determine the treatment from the schedule."

ROUTE COST QUESTIONS

"I will include your question about the treatment estimate."

CLARIFY TIMING

"The allocated thirty minutes does not guarantee a procedure today."

PRESERVE THE CONCERN

"I will tell the dentist that you expected treatment today."

SEPARATE ATTENDANCE AND CONSENT

"Coming to this visit is not agreement to every possible procedure."

QUALIFY COVERAGE

"Any coverage estimate needs confirmation through the appropriate process."

CHECK UNDERSTANDING

"Are we clear that the current booking is for consultation?"

CLOSE THE HANDOFF

"We will clarify the plan and your questions before treating a procedure as agreed."

Cannot determine

Keeps an administrative record separate from diagnosis.

Include your question

Routes the concern without abandoning it.

Allocated thirty minutes

Describes time reserved, not treatment readiness or outcome.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which response is accurate?

- A** You arrived, so you consented to treatment.
- B** The booking proves which tooth needs repair.
- C** The record shows a consultation; the dentist can clarify the plan before you decide.
- D** A thirty-minute slot guarantees a filling.

2. Which statement overpromises?

- A** The dentist can address your question.
- B** Your insurance will definitely pay because the visit is booked.
- C** The estimate needs checking.
- D** No procedure is confirmed yet.

3. What should be handed to the dentist?

- A** An invented diagnosis
- B** A statement that all treatment is already accepted
- C** A changed record hiding the original booking
- D** The mismatch between the expected filling and recorded consultation, plus the patient's questions

4. What does attendance establish by itself?

- A** Confirmation of clinical suitability
- B** Acceptance of every future cost
- C** The patient came to the appointment, not agreement to every procedure
- D** Consent to every possible treatment

[Answers and explanations: page 85.](#)

A consultation is not a confirmed filling

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

booking discrepancy / coverage estimate / treatment estimate / decision time / Informed consent / patient expectation / treatment plan / appointment duration / chief concern / appointment type

- 01 **Lena:** I am here for the filling. I took the afternoon off, so I hope we can finish it today rather than arrange another visit.
- 02 **Amir:** I can see why that is frustrating. I will flag the ¹ _____ to the dentist: you came for a filling, but our schedule shows a thirty-minute consultation. No procedure is confirmed yet.
- 03 **Lena:** When I spoke to reception, I mentioned the tooth that was bothering me. I assumed booking an appointment meant booking the treatment as well.
- 04 **Amir:** The ² _____ matters here. A consultation lets the dentist discuss the concern and appropriate next steps, but I should not tell you that it guarantees a filling today.
- 05 **Lena:** Does the shorter appointment mean the dentist has already decided there is nothing wrong? I do not want my concern dismissed before anyone looks.
- 06 **Amir:** No. The ³ _____ does not establish a diagnosis or the seriousness of your concern. I will tell the dentist what you expected and ask them to clarify the purpose with you.
- 07 **Lena:** My main question is whether this tooth needs a filling at all. I also want to know what the alternatives would involve.
- 08 **Amir:** I can pass on that ⁴ _____. The dentist needs to assess and explain the options; I cannot determine the treatment from a booking entry or from where you point.
- 09 **Lena:** I would also like to understand the cost before agreeing. I received a number over the phone, but I do not know exactly what it covered.
- 10 **Amir:** We should clarify the ⁵ _____ and which service it described. A consultation estimate and an estimate for a particular restoration may not cover the same work.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 10.

- 11 **Lena:** The receptionist also mentioned my insurer. Should I assume that the amount above that estimate will be paid by insurance?
- 12 **Amir:** No. A ⁶ _____ is not a payment guarantee. I can route the question to the appropriate person so the limits are clear before you rely on it.
- 13 **Lena:** If I stay to talk to the dentist, am I agreeing to go ahead with whatever they recommend during that conversation?
- 14 **Amir:** No. ⁷ _____ concerns the actual care being proposed and discussed. Attending this visit does not mean you have already accepted every possible procedure or cost.
- 15 **Lena:** Thank you. I want the dentist to know I expected treatment, but I do not want anyone to think I am demanding a procedure that might not be right.
- 16 **Amir:** I will describe the ⁸ _____ neutrally. Your expectation and your request to understand the options can both be included, without presenting either as a clinical decision.
- 17 **Lena:** Then I would like to ask the dentist to explain what can be decided today and whether another appointment might be needed.
- 18 **Amir:** That is a useful question about the ⁹ _____. I will make sure it reaches the dentist rather than promise that today's allocated time can accommodate any treatment that may be recommended.
- 19 **Lena:** All right, I will speak to the dentist first. Please mention the time off work too. Another appointment would take some arranging.
- 20 **Amir:** I will. We will allow ¹⁰ _____ for that discussion and keep the current status clear: consultation booked, procedure unconfirmed, and your questions still to be answered.

ANSWERS / EXPLANATIONS: PAGE 86

1. patient expectation | 2. appointment type | 3. appointment duration | 4. chief concern | 5. treatment estimate | 6. coverage estimate | 7. Informed consent | 8. booking discrepancy | 9. treatment plan | 10. decision time

Clarify a review rather than a procedure booking

F Say it

A patient expects a crown fitting. The schedule shows a twenty-minute review, and the dentist has not confirmed fitting readiness. The patient asks about timing and the estimate.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

unconfirmed / twenty / review / crown fitting

Assistant: "The recorded appointment type is a ¹ _____."

Patient: "I expected a ² _____."

Assistant: "The allocated time is ³ _____ minutes."

Assistant: "Fitting readiness remains ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 86

1. review | 2. crown fitting | 3. twenty | 4. unconfirmed

Rehearse with a fixed script

1. Read Lena and Amir aloud; stress consultation and no procedure confirmed.
2. Repeat the exchange with the crown-fitting facts below. Keep the twenty-minute duration separate from readiness.
3. Check that neither speaker promises treatment or insurance payment; use the numbered answers to correct the four blanks.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

The pack is present; readiness is not verified

A Read the situation

Raise an unresolved instrument-readiness check clearly and seek a verified replacement without waiving the practice process.

Assistant Tessa sees instrument pack P18 in the treatment room. The required readiness documentation cannot be confirmed, and the dentist is about to begin. Tessa has not released the pack for use and is not authorized to waive the practice checks. Lead assistant Bruno can investigate and may have a verified replacement. No contamination or sterilization failure has been established in the supplied facts. The immediate communication task is to make the unresolved check visible, keep the pack from being used until properly resolved, and follow the actual practice procedure.

PEOPLE / Tessa: Dental assistant | Bruno: Lead assistant

A CLEAR INTERRUPTION CAN PROTECT THE WORKFLOW

A junior worker may hesitate when the team is ready to begin. Name the specific unresolved check and the needed action. A factual concern is not a personal accusation, and politeness should not make a request to pause sound optional.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What is known about P18?

- | | |
|---|--|
| A Every required check has passed. | B It was already used on the patient. |
| C It is present, but required readiness documentation cannot be confirmed. | D It has been proved contaminated. |

2. What may Bruno provide?

- | | |
|---|--|
| A A guaranteed replacement before checking | B A replacement whose readiness has been verified |
| C An automatic waiver for every pack | D Permission inferred from the busy schedule |

3. What must not happen?

- | | |
|--|---|
| A Reporting the gap | B Asking for clarification |
| C Identifying P18 in the record | D Treating pack presence as permission to skip the required checks |

Answers and explanations: page 87.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

instrument pack

A packaged set of dental instruments with a defined processing and use status.

IN USE identify the instrument pack

reprocessing

The required sequence for preparing a reusable device for further use.

IN USE follow reprocessing instructions

decontamination

Removal or reduction of contamination through the applicable process.

IN USE follow decontamination procedures

cleaning

Removal of soil and material, distinct from sterilization.

IN USE verify the cleaning step

disinfection

A process reducing or inactivating specified microorganisms under defined conditions.

IN USE distinguish disinfection from sterilization

sterilization

A validated process intended to eliminate viable microorganisms, including spores.

IN USE verify sterilization records

sterilizer

Equipment used for an appropriate sterilization process.

IN USE identify the sterilizer

cycle record

Documentation of a particular processing cycle.

IN USE trace the cycle record

load number

The identifier linking items to a particular processing load.

IN USE verify the load number

mechanical monitoring

Checking process parameters such as time, temperature, and pressure as applicable.

IN USE review mechanical monitoring

chemical indicator

A material showing exposure to specified process conditions.

IN USE inspect the chemical indicator

biological indicator

A test system using resistant microorganisms to assess the sterilization process.

IN USE review biological indicator results

Precision check

Present, packaged, clean, disinfected, and sterile are not interchangeable statuses. Recognition of an indicator or a sealed pack does not replace the required combined checks, records, and manufacturer directions. This unit practices clarification, not a processing protocol.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

pack integrity

The condition of the packaging and its protective barrier.

IN USE check pack integrity

compromised packaging

Packaging whose protective condition has been damaged or otherwise impaired.

IN USE report compromised packaging

IFU

Instructions for use; the relevant manufacturer's directions.

IN USE consult the IFU

single-use device

A device intended for one use under its labeling.

IN USE identify a single-use device

reusable device

A device intended for further use after the specified reprocessing.

IN USE identify a reusable device

readiness documentation

Records needed to establish the relevant item is ready under the procedure.

IN USE confirm readiness documentation

traceability

Ability to connect an item with relevant identity and processing history.

IN USE maintain traceability

release status

Whether an item is authorized for use under the required checks.

IN USE confirm release status

hold

A status preventing use pending the appropriate resolution.

IN USE maintain the hold

verified replacement

An alternative item whose required readiness has been confirmed.

IN USE obtain a verified replacement

processing deviation

A departure from the specified processing requirement.

IN USE report a processing deviation

infection prevention

Measures intended to reduce the risk of infection transmission.

IN USE follow infection prevention procedures

Precision check

Cannot confirm readiness is more accurate than confirmed contaminated when no failure has been established. The distinction does not justify use. Keep the unresolved status visible and follow the actual hold, review, replacement, and release process.

Phrases you can actually use

C Notice the language / 1 of 2

INTERRUPT CLEARLY

"Please pause; I cannot confirm the required readiness check for P18."

STATE THE GAP

"The required documentation is not available for verification."

PREVENT AN ASSUMPTION

"The pack being present does not establish its release status."

REQUEST THE LEAD

"Can you review the status through the practice procedure?"

IDENTIFY THE ITEM

"The concern is about pack P18 in this room."

AVOID AN UNSUPPORTED FINDING

"I am not reporting a confirmed contamination event."

NAME THE LIMIT

"I am not authorized to waive this check."

ASK ABOUT AN ALTERNATIVE

"Is a verified replacement available?"

Please pause

Makes the immediate action clear without blaming a colleague.

Cannot confirm

Identifies missing verification rather than inventing a result.

Not authorized to waive

Names the actual authority boundary.

Phrases you can actually use

C Notice the language / 2 of 2

KEEP A HOLD VISIBLE

"This pack should not be used while the required check remains unresolved."

CLARIFY THE EVIDENCE

"Which record confirms the relevant processing and release checks?"

CONFIRM THE REPLACEMENT

"Please confirm the replacement identifier and verified status."

RECORD THE ACTION

"I will record the concern and the action actually taken."

PRESERVE TRACEABILITY

"Keep the pack identifier linked to the review record."

REJECT SCHEDULE PRESSURE

"The appointment timing does not complete the missing check."

SEPARATE TWO OUTCOMES

"Replacing the pack does not by itself resolve the original documentation issue."

CLOSE THE LOOP

"We need an explicit readiness confirmation before describing the setup as ready."

Verified replacement

Requires readiness as well as physical availability.

By itself

Prevents a workaround from being mistaken for resolution of the original issue.

Explicit confirmation

Avoids assuming that silence or activity means release approval.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which message is best?

- A** The pack looks clean, so it must be ready.
- B** Every instrument is contaminated, without evidence.
- C** We can waive the check because the dentist is waiting.
- D** Please pause; P18's required readiness record is unconfirmed, and I need the approved review or a verified replacement.

2. What does a color change alone establish here?

- A** That pack integrity no longer needs checking
- B** Not completion of every required readiness check
- C** That the pack's release record is complete
- D** That the biological monitoring result has passed

3. What should happen to the original issue if a replacement is found?

- A** It automatically disappears from all records.
- B** It proves P18 was contaminated.
- C** It permits changing the old record to show prior verification.
- D** It remains subject to the required review and record process.

4. Which statement is precise?

- A** Missing records prove every cycle failed.
- B** No documented failure means automatic permission to use the pack.
- C** Readiness is unverified; no confirmed failure is established in the supplied facts.
- D** Readiness and contamination are identical words.

Answers and explanations: page 87.

The pack is present; readiness is not verified

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

cycle record / chemical indicator / verified replacement / instrument pack / load number / IFU / Traceability / hold / release status / readiness documentation

- 01 **Tessa:** Bruno, please pause the setup. Pack P18 is present, but I cannot confirm its required readiness documentation, and the dentist is about to begin.
- 02 **Bruno:** P18, understood. Has that ¹ _____ been opened or used? Tell me where it is now so nobody picks it up while I check the record.
- 03 **Tessa:** It has not been used. I have not released it, and I do not have authority to waive the missing check just because the room is busy.
- 04 **Bruno:** Then keep the ² _____ unresolved under our procedure. I will help review the record rather than treat the presence of the pack as proof that it is ready.
- 05 **Tessa:** The package is here, but I do not want to say it is contaminated. The issue I can establish is that the required documentation cannot be confirmed.
- 06 **Bruno:** That is the right distinction. We need the ³ _____, not an invented finding of failure or a reassuring assumption. The uncertainty still means we must not bypass the check.
- 07 **Tessa:** Would a replacement solve the immediate setup problem? I heard there might be another pack, but I have not checked its identity or status.
- 08 **Bruno:** A ⁴ _____ may be available. I will check the actual alternative and its required status, not simply swap one unverified package for another because it is nearby.
- 09 **Tessa:** If we locate the original record, we also need to know that it belongs to P18 rather than a similar pack from another processing load.
- 10 **Bruno:** Correct. ⁵ _____ connects the item to the relevant history. A complete-looking record for a different load would not answer the question about this pack.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 19.

- 11 **Tessa:** I can give you the identifiers shown without altering anything. I do not want to copy a number from another pack into the missing record.
- 12 **Bruno:** Preserve the actual ⁶ _____ and follow the documentation process. A guessed identifier would hide the problem rather than establish that the required processing and checks occurred.
- 13 **Tessa:** Someone asked whether the visible indicator was enough. I was unsure how to explain why one visible feature did not settle every readiness question.
- 14 **Bruno:** A ⁷ _____ has a defined purpose, but it is not a substitute for all applicable checks. We need the practice process and manufacturer instructions followed, not an isolated shortcut.
- 15 **Tessa:** I will tell the dentist that I have raised the specific gap and that you are checking the record and a possible verified alternative.
- 16 **Bruno:** Keep the ⁸ _____ visible. I am checking the replacement now; please tell the dentist that setup is still waiting for verification, not ready yet.
- 17 **Tessa:** If another pack is confirmed, should I describe the entire issue as closed, or distinguish the replacement from the original record question?
- 18 **Bruno:** Distinguish them. The ⁹ _____ issue may require further review even after the immediate setup has an acceptable alternative. Do not erase the first concern from the history.
- 19 **Tessa:** Understood. I will record the actual pack status, whom I contacted, and any verified replacement details, without claiming a failure that has not been found.
- 20 **Bruno:** And follow the relevant ¹⁰ _____ and practice procedures for the item. Our handoff should end with explicit confirmed facts, not with the assumption that someone else must have completed the check.

ANSWERS / EXPLANATIONS: PAGE 88

1. instrument pack | 2. release status | 3. readiness documentation | 4. verified replacement | 5. Traceability | 6. load number | 7. chemical indicator | 8. hold | 9. cycle record | 10. IFU

Identify a different unresolved readiness check

F Say it

Pack Q7 has not been used. Its required readiness check is unconfirmed. Lead assistant Eva can review it; a replacement has been requested but not yet verified.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

verified / Eva / not used / Q7

Assistant: "The affected pack is ¹ _____."

Lead: "Its use status is ² _____."

Assistant: "The review contact is ³ _____."

Lead: "The replacement is requested but not yet ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 88

1. Q7 | 2. not used | 3. Eva | 4. verified

Rehearse with a fixed script

1. Read the pause request aloud with P18 and the missing readiness record clearly audible.
2. Repeat the four-line transfer using its different pack and check; do not call an unresolved check a confirmed contamination event.
3. Check the answers, then repeat the difference between a verified replacement and resolution of the original record issue.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Which tray did you mean?

A Read the situation

Identify the word you missed, request the distinguishing detail, and confirm the full chairside instruction before acting.

During a setup briefing, assistant Noor hears Dr. Shaw say "tray" but misses the rest because another conversation interrupts. The setup list contains both an instrument tray and an impression tray. Noor has not selected either. She needs to ask which item is intended and read back the complete request before continuing under practice procedures. The dentist then clarifies that the instrument tray is wanted for the current setup. Knowing the general treatment context does not justify guessing the missing instruction, and this language exercise does not authorize a clinical procedure.

PEOPLE / Noor: Dental assistant | Dr. Shaw: Dentist

A BRIEF CLARIFICATION IS PROFESSIONAL

Fast speech and local shorthand can make an unfamiliar term seem obvious to everyone else. State exactly what you heard and what you missed. A precise question is more useful than a vague yes, an apology repeated several times, or a silent guess.

Briefing checks

Circle one answer in each item. Use the briefing.

1. Which word did Noor hear?

- A A confirmed size
- C Instrument

- B Tray
- D Impression

2. What are the two listed possibilities?

- A Instrument tray and impression tray
- C A radiograph and a consent form

- B Two confirmed identical instruments
- D Two appointment times

3. What does the dentist clarify?

- A Noor should guess each future request.
- C Every clinical check is waived.

- B The impression tray was already selected.
- D The instrument tray is wanted for the current setup.

Answers and explanations: [page 89](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

chairside

Relating to assistance beside the dental chair during care.

IN USE provide chairside support

setup briefing

A discussion confirming the planned equipment and arrangements.

IN USE conduct a setup briefing

instrument tray

A tray used to organize the relevant dental instruments.

IN USE confirm the instrument tray

impression tray

A device used to hold impression material under the appropriate procedure.

IN USE identify the impression tray

impression

A negative reproduction used in making a model of oral structures.

IN USE distinguish an impression from a scan

intraoral scan

A digital capture of structures inside the mouth.

IN USE identify the intraoral scan

mirror

A dental instrument used for viewing or related clinical purposes.

IN USE identify the requested mirror

explorer

A dental examination instrument used by appropriately trained personnel.

IN USE identify the explorer

periodontal probe

An instrument for periodontal measurements under the relevant clinical procedure.

IN USE distinguish the periodontal probe

cotton pliers

An instrument for handling suitable small materials.

IN USE identify the cotton pliers

HVE

High-volume evacuation; a dental suction system or device.

IN USE clarify the HVE request

saliva ejector

A suction device intended for removing saliva under its specified use.

IN USE distinguish the saliva ejector

Precision check

Instrument tray and impression tray share a word but identify different items. Repeat the distinguishing term, not only tray. The same principle applies to size, identifier, side, and timing when those details are part of the actual request.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

air-water syringe

A dental device delivering air, water, or a combination under its design.

IN USE identify the air-water syringe

handpiece

A powered dental instrument used with specified attachments.

IN USE clarify the handpiece type

bur

A rotary cutting or finishing attachment with a particular design and purpose.

IN USE verify the bur identifier

matrix band

A device used to provide a temporary wall during certain restorations.

IN USE identify the matrix band

wedge

A small device used for specified separation or adaptation tasks in dentistry.

IN USE clarify the requested wedge

curing light

A device used to initiate setting of compatible dental materials.

IN USE identify the curing light

retractor

An instrument or device used to hold tissue away as appropriate.

IN USE identify the retractor

distinguishing detail

The feature that separates one possible request from another.

IN USE request the distinguishing detail

read-back

Repeating an instruction to check it was heard correctly.

IN USE give a complete read-back

closed-loop request

A request followed by acknowledgment and confirmation of the intended information.

IN USE complete a closed-loop request

ambiguous instruction

Wording that supports more than one interpretation.

IN USE clarify an ambiguous instruction

interruption

Something that breaks attention or prevents hearing the full message.

IN USE acknowledge an interruption

Precision check

Recognizing a term does not demonstrate competence to use the equipment. These definitions support listening and clarification. Follow the actual procedure, training, and authorization; a successful read-back confirms communication, not every separate readiness or safety requirement.

Phrases you can actually use

C Notice the language / 1 of 2

03

SIGNAL THE GAP

"I heard tray, but I missed the word before it."

NAME THE INTERRUPTION

"Another conversation covered the rest of the request."

OFFER THE ALTERNATIVES

"Did you mean the instrument tray or the impression tray?"

AVOID GUESSING

"I have not selected either yet."

ASK FOR THE WHOLE REQUEST

"Could you repeat the full item name?"

READ BACK

"The instrument tray for the current setup, correct?"

CHECK A MODIFIER

"Which size or identifier did you specify?"

CLARIFY SEQUENCE

"Do you need that now or at the next stage?"

I heard, but I missed

Separates the reliable part of the message from the missing part.

Did you mean X or Y

Narrows a genuine ambiguity to the stated alternatives.

Not selected either

Confirms that no guess has already become an action.

Phrases you can actually use

C Notice the language / 2 of 2

03

CONFIRM A CORRECTION

"Thank you; I will use the corrected request."

KEEP CHECKS SEPARATE

"Hearing the request correctly does not replace the readiness checks."

REQUEST ORDINARY WORDING

"Could you say the full name once?"

PROTECT THE HANDOFF

"I will pass on the complete request rather than the shortened version."

AVOID VAGUE AGREEMENT

"I need to confirm the item before saying it is ready."

NAME UNFAMILIAR SHORTHAND

"I do not know that abbreviation in this practice."

STATE WHAT IS CONFIRMED

"We have confirmed the instrument tray, not the impression tray."

CLOSE THE LOOP

"I have repeated the item, and you have confirmed it."

Correct?

Invites explicit confirmation of the repeated instruction.

Full name once

Requests efficient clarification without a lengthy apology.

Does not replace

Keeps communication confirmation distinct from other required checks.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which question best resolves the actual ambiguity?

- A** Can I ignore the rest because I heard one word?
- B** Did you mean the instrument tray or the impression tray?
- C** You said tray, so either must be acceptable.
- D** Shall I choose the one nearest me?

2. Which read-back is complete for the clarified request?

- A** The instrument tray for the current setup, correct?
- B** The tray from yesterday, correct?
- C** The impression tray for the current setup, correct?
- D** The instrument tray for the next appointment, correct?

3. What should Noor do with unfamiliar local shorthand?

- A** Pretend every abbreviation is universal.
- B** Choose an item based only on its color.
- C** Change the request without telling anyone.
- D** Ask for the full name and confirm the intended meaning.

4. What does confirmation of the request establish?

- A** A waiver of the practice procedure
- B** Consent for an unrelated treatment
- C** Shared understanding of the requested item, not completion of all readiness checks
- D** Universal competence to use all devices

Answers and explanations: [page 89](#).

Which tray did you mean?

E Conversations / turns 1-10

03

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

read-back / impression tray / closed-loop request / ambiguous instruction / interruption / handoff / setup briefing / instrument tray / distinguishing detail / chairside

- 01 **Noor:** Dr. Shaw, I heard tray, but the conversation by the door covered the first word. Instrument tray or impression tray? I have not selected either.
- 02 **Dr. Shaw:** Sorry about the ¹ _____. The instrument tray for this setup, please. The impression tray is not part of this request.
- 03 **Noor:** Instrument tray for the current setup, correct? I will check its readiness status before bringing it over. The other tray will stay where it is.
- 04 **Dr. Shaw:** Correct; that ² _____ matches. Also, have the high-volume evacuator ready under our normal checks. We will confirm the remaining equipment during the briefing.
- 05 **Noor:** I heard suction from the doorway earlier. Do you mean the HVE rather than the saliva ejector? Both are on the equipment list.
- 06 **Dr. Shaw:** Yes, HVE. That is the ³ _____ I left out earlier. Please keep those names separate when you pass the request to Marta.
- 07 **Noor:** HVE confirmed. Marta has asked which tray to bring. I will give her the full item name and tell her this is for the current setup.
- 08 **Dr. Shaw:** Ask her for the ⁴ _____, with the required readiness checks completed. If she cannot confirm its status, have her tell us before it reaches the setup.
- 09 **Noor:** One more point: someone put small suction on the handwritten list. Is that your request, or a note carried over from the previous appointment?
- 10 **Dr. Shaw:** I cannot tell from that note. Treat it as an ⁵ _____, not a substitute for the HVE request we have just confirmed. I will check its source.

The conversation, continued

E Conversations / turns 11-20

03

Continue with the word bank on page 28.

- 11 **Noor:** I will mark that question as unresolved. I will not remove an equipment requirement merely because the handwriting is unclear. We still need to establish what it refers to.
- 12 **Dr. Shaw:** During the ⁶ _____, we will reconcile that note with the actual list. Please bring the original wording so I can see what needs clarification.
- 13 **Noor:** For any size-specific request, please include the size in the full name. I missed a modifier in yesterday's briefing and had to ask again.
- 14 **Dr. Shaw:** Of course. In a ⁷ _____ exchange, say which part you missed. I would rather repeat one size than discover that we understood different items.
- 15 **Noor:** Marta is here now. She has repeated instrument tray and HVE. She also wants to confirm that you did not request impression equipment at this stage.
- 16 **Dr. Shaw:** Correct: no ⁸ _____ in this request. That is about our current setup, not a change to another patient's appointment or planned equipment.
- 17 **Noor:** I have repeated that to Marta, and she has confirmed it back. She is checking the tray's readiness and will report before bringing it across.
- 18 **Dr. Shaw:** That completes the ⁹ _____. Let me know if the check is unresolved; an accurate equipment name does not settle its processing status.
- 19 **Noor:** My note reads: instrument tray and HVE for this setup, checks required; small suction note awaiting clarification. Marta has those same details.
- 20 **Dr. Shaw:** That is the ¹⁰ _____ I need. Bring the handwritten list to the briefing, and we will settle the remaining point before relying on it.

ANSWERS / EXPLANATIONS: PAGE 90

1. interruption | 2. read-back | 3. distinguishing detail | 4. instrument tray | 5. ambiguous instruction | 6. setup briefing | 7. chairside | 8. impression tray | 9. closed-loop request | 10. handoff

Clarify a suction-device request

F Say it

During a briefing, an assistant hears suction but misses the device name. The list includes an HVE device and a saliva ejector. The dentist clarifies HVE for the current setup; normal checks remain required.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

device name / HVE / required / read-back

Assistant: "I heard suction but missed the ¹ _____."

Dentist: "The requested device is ² _____."

Assistant: "I will confirm it with a complete ³ _____."

Dentist: "The ordinary readiness checks remain ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 90

1. device name | 2. HVE | 3. read-back | 4. required

Rehearse with a fixed script

1. Read the chairside briefing in pairs; pause after each read-back for the confirming reply.
2. Repeat the suction-device transfer, clearly distinguishing HVE from saliva ejector.
3. Use the key to correct the four blanks, then repeat the final line without implying that identification completes readiness checks.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Explain the route to an answer, not the image

A Read the situation

Acknowledge a question about dental imaging and arrange clinical explanation without interpreting findings or promising treatment.

Patient Marco looks at a dental image and asks assistant Suki whether it proves that a tooth needs treatment. The dentist has not yet discussed the image with him. Suki is assisting with the visit and is not authorized to interpret the image or recommend treatment. She can identify the question, explain her role, and ask the dentist to discuss the findings and their significance. No finding, diagnosis, or treatment decision is supplied in this case. Technical vocabulary supports communication; it does not qualify a learner to read an image.

PEOPLE / Marco: Patient | Suki: Dental assistant

TECHNICAL CONFIDENCE SHOULD NOT BECOME A DIAGNOSIS

A patient may expect any team member near an image to explain it. Acknowledge the specific question and make the route to an answer clear. Avoid casual reassurance or an alarming guess based on an apparent shadow, color, or shape.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What does Marco ask?

A Whether the image proves that a tooth needs treatment

C Which pack was processed

B Whether the laboratory confirmed delivery

D Whether his appointment was cancelled

2. What is Suki not authorized to do here?

A Preserve the patient's concern

C Acknowledge the question

B Interpret the image or recommend treatment

D Ask the dentist to explain

3. What clinical finding is established in the brief?

A A definite need for extraction

C A confirmed cavity

B None

D A confirmed fracture

Answers and explanations: page 91.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

radiograph

An image produced using X-rays.

IN USE identify the radiograph

radiography

The process of producing radiographic images under the applicable rules.

IN USE distinguish radiography from interpretation

intraoral image

In dental radiography, an image obtained with the receptor inside the mouth.

IN USE identify an intraoral image

extraoral image

In dental radiography, an image obtained with the receptor outside the mouth.

IN USE identify an extraoral image

bitewing

A dental radiographic view commonly showing crowns of upper and lower teeth and nearby structures.

IN USE identify the bitewing view

periapical image

A dental image showing a tooth and the area around its root tip.

IN USE identify a periapical image

panoramic image

A broad dental image of the jaws and surrounding structures.

IN USE identify a panoramic image

CBCT

Cone-beam computed tomography; three-dimensional X-ray imaging used for appropriate indications.

IN USE identify the CBCT study

image receptor

The film or digital sensor receiving the image information.

IN USE identify the image receptor

field of view

The area or volume included in an imaging examination.

IN USE clarify the field of view

exposure

The use of radiation to obtain an image under an authorized process.

IN USE document the exposure

image quality

Whether an image meets the relevant technical requirements for its intended use.

IN USE assess image quality within role

Precision check

Acquiring, labeling, displaying, and interpreting an image are different activities. The actual role and local rules determine who may do each. A visible feature or a technical term does not establish a diagnosis or make an assistant authorized to provide one.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

artifact

An image feature introduced by a technical or other non-anatomical factor.

IN USE report a possible artifact

radiolucent

Appearing relatively darker on a radiograph because more X-rays reach the receptor.

IN USE recognize the term radiolucent

radiopaque

Appearing relatively lighter on a radiograph because fewer X-rays reach the receptor.

IN USE recognize the term radiopaque

anatomical structure

A part of the body represented or examined.

IN USE identify the named anatomical structure

image interpretation

Professional assessment of what an image shows and means.

IN USE refer image interpretation

clinical finding

Information established through the appropriate clinical assessment.

IN USE explain a clinical finding

diagnostic significance

The relevance of information to identifying a condition.

IN USE ask about diagnostic significance

correlation

Considering information together with other relevant findings.

IN USE correlate clinical information

comparison image

An earlier or other image used for a defined comparison.

IN USE identify the comparison image

image annotation

A label or mark added to an image with its meaning and source identified.

IN USE clarify an image annotation

treatment indication

The clinical reason for considering a particular treatment.

IN USE explain the treatment indication

authorized interpreter

A professional permitted and qualified to interpret the relevant image.

IN USE refer to the authorized interpreter

Precision check

An image may contribute to a clinical assessment, but the learner is not given a finding to interpret here. Preserve the question about treatment need and ask the dentist to explain the evidence, relevant uncertainty, and options in understandable terms.

Phrases you can actually use

C Notice the language / 1 of 2

ACKNOWLEDGE THE QUESTION

"You want to know whether the image means this tooth needs treatment."

OFFER THE NEXT STEP

"I will ask the dentist to explain the findings with you."

AVOID FALSE REASSURANCE

"I should not tell you everything is normal without the clinical explanation."

PRESERVE THE EXACT CONCERN

"I will include your question about whether treatment is necessary."

STATE THE ROLE

"I am not authorized to interpret the image."

AVOID A VISUAL GUESS

"I cannot identify a diagnosis from that feature for you."

SEPARATE PROCESS AND MEANING

"Helping with an image is different from interpreting it."

ASK FOR ORDINARY LANGUAGE

"Could the dentist explain that term in everyday language?"

You want to know whether

Restates the substance instead of answering a different question.

Not authorized to interpret

Identifies the specific boundary without denying the patient's right to an explanation.

How it relates

Invites an explanation of the whole clinical assessment.

Phrases you can actually use

C Notice the language / 2 of 2

CLARIFY AN ANNOTATION

"Please ask the clinician what that marked area refers to."

INVITE EVIDENCE QUESTIONS

"You can ask how the image relates to the rest of the examination."

AVOID OVERSTATING PROOF

"I cannot say the image alone proves a particular treatment is needed."

CHECK THE HANDOFF

"I will make sure the question reaches the dentist before you decide."

KEEP DECISIONS OPEN

"No treatment decision has been explained in this conversation yet."

DISTINGUISH ALTERNATIVES

"You can ask which options are being considered and why."

ROUTE A TECHNICAL QUESTION

"The clinician can explain the significance of that imaging term."

CLOSE WITHOUT ABANDONMENT

"I cannot give the interpretation, but I can help you get a clear answer."

No decision explained yet

Describes the current conversation without claiming no clinical assessment exists elsewhere.

In everyday language

Requests accessible explanation rather than less accurate information.

Before you decide

Connects understanding with the patient's actual choice.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which response is appropriate?

- A** Everything is normal because you are not in pain.
- B** Any image means treatment is compulsory.
- C** I cannot interpret it, but I will ask the dentist to explain whether it supports treatment and why.
- D** The dark area definitely proves decay.

2. What is wrong with labeling an unexplained feature a cavity?

- A** The patient has already received an explanation.
- B** The schedule supplies diagnostic proof.
- C** No interpreted finding is supplied, and Suki lacks authorization to diagnose it.
- D** Every dark feature always means the same thing.

3. Which distinction is necessary?

- A** Producing an image and interpreting it are different activities.
- B** Displaying the image confirms the diagnosis recorded elsewhere.
- C** Correctly positioning a receptor establishes authority to recommend treatment.
- D** Knowing the term radiolucent establishes what this feature represents.

4. Which question should reach the dentist?

- A** Can the assistant invent a reassuring result?
- B** Can all uncertainty be deleted from the record?
- C** Can an annotation replace the explanation?
- D** Does the image support treatment, and how does it relate to the examination and options?

Answers and explanations: [page 91](#).

Explain the route to an answer, not the image

E Conversations / turns 1-10

04

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

authorized interpreter / radiograph / image interpretation / diagnostic significance / treatment indication / comparison image / radiolucent / artifact / correlation / image annotation

- 01 **Marco:** Suki, I can see a darker area on this picture. Does that prove the tooth needs treatment, or am I misunderstanding what I am looking at?
- 02 **Suki:** You are asking about ¹ _____. I am not authorized to interpret the image, but I will ask the dentist to explain the finding, if any, and how it relates to treatment.
- 03 **Marco:** I thought you could explain it because you displayed it. Who should I ask about that darker patch?
- 04 **Suki:** Displaying a ² _____ and interpreting it are different tasks. I can help communicate the question without giving you a diagnosis from something I see on the display.
- 05 **Marco:** Is darker always worse? I have heard words used for dark and light areas, but I do not know whether they describe a disease.
- 06 **Suki:** Terms such as ³ _____ describe image appearance, not a diagnosis by themselves. The dentist can explain the relevant terminology in context rather than have us attach a condition to an isolated word.
- 07 **Marco:** Then I should ask what the feature actually represents. I would also like to know whether anything else in the examination supports the same conclusion.
- 08 **Suki:** That is a question about ⁴ _____ with other clinical information. I will pass it on along with your question about whether a treatment recommendation is supported and what alternatives exist.
- 09 **Marco:** There is also a mark on the screen. I cannot tell whether it is part of the original image or something someone added to point out an area.
- 10 **Suki:** The clinician can clarify that ⁵ _____ and what it refers to. I should not guess the purpose of a mark or present it as proof of a particular condition.

The conversation, continued

E Conversations / turns 11-20

04

Continue with the word bank on page 37.

- 11 **Marco:** Could it be a technical issue instead? I am not asking you to decide that, but I would like the dentist to address the possibility.
- 12 **Suki:** I will include your question about a possible ⁶ _____. We should keep that as a question, not replace one unsupported conclusion with another.
- 13 **Marco:** I appreciate that. I do not want reassurance if it is not justified, but I also do not want a frightening answer based on a guess.
- 14 **Suki:** The ⁷ _____ can explain what is established and what remains uncertain. I can make sure your concern is heard without telling you that everything is normal or definitely abnormal.
- 15 **Marco:** If a treatment is recommended, I want to understand why that treatment follows from the evidence rather than think the picture makes the decision automatically.
- 16 **Suki:** I will ask the dentist to explain the ⁸ _____ in ordinary language. An image on a screen is not a substitute for the discussion you need before deciding.
- 17 **Marco:** Would an earlier image matter? Has the dentist compared this with my previous image?
- 18 **Suki:** You can ask whether a ⁹ _____ was relevant and reviewed. I will not say that comparison has happened unless it is confirmed in the actual explanation.
- 19 **Marco:** Please ask what the patch means and whether it matches the examination. That is what I need to understand first.
- 20 **Suki:** I will preserve them for the ¹⁰ _____ discussion. We have identified the questions, not a diagnosis or an agreed procedure, and I will help you obtain the clinical explanation.

ANSWERS / EXPLANATIONS: PAGE 92

1. diagnostic significance | 2. radiograph | 3. radiolucent | 4. correlation | 5. image annotation | 6. artifact | 7. authorized interpreter | 8. treatment indication | 9. comparison image | 10. image interpretation

Route a question about an imaging term

F Say it

04

A patient asks whether a note saying radiopaque proves that a tooth needs a crown. The assistant is not authorized to interpret the image. The dentist has not yet explained the note.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

established / dentist / crown / radiopaque

Assistant: "The term in your question is ¹ _____."

Patient: "I want to know whether I need a ² _____."

Assistant: "The explanation needs the ³ _____."

Assistant: "No treatment conclusion has been ⁴ _____ here."

ANSWERS / EXPLANATIONS: PAGE 92

1. radiopaque | 2. crown | 3. dentist | 4. established

Rehearse with a fixed script

1. Read Marco and Suki aloud; keep radiolucent distinct from a diagnosis.
2. Repeat the transfer with radiopaque and crown. Preserve the question rather than confirming a need for treatment.
3. Check all four answers, then read the corrected handoff to the dentist aloud.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Reassure without promising a painless procedure

A Read the situation

Acknowledge dental anxiety, communicate a comfort concern, and request an agreed pause signal without guaranteeing sensations.

Patient Ava tells assistant Ben she is anxious and asks him to promise that the planned procedure will not hurt. The dentist has not yet discussed comfort measures or her concerns. Ben can listen, relay the concern, and ask the dentist to explain the plan and an appropriate pause signal before proceeding. He cannot guarantee a painless experience, select medication, or invent a sedation arrangement. No specific treatment or anesthetic plan is supplied. The conversation should help Ava ask questions and understand how concerns will be addressed within the actual care process.

PEOPLE / Ava: Patient | Ben: Dental assistant

REASSURANCE SHOULD BE HONEST AND SPECIFIC

Phrases such as you will not feel a thing can sound kind but promise more than the speaker knows. Acknowledge the concern without minimizing it. Explain the next conversation and let the clinician discuss likely sensations, options, and the way to request a pause.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What does Ava ask Ben to promise?

- | | |
|---|---|
| A That the laboratory will deliver early | B That the visit will be free |
| C That a particular sedative is prescribed | D That the procedure will not hurt |

2. What has not happened yet?

- | | |
|--|------------------------------------|
| A The dentist's discussion of comfort measures and Ava's concerns | B Any expression of anxiety |
| C Ava's arrival | D Ben hearing the request |

3. What can Ben appropriately arrange?

- | | |
|--|--|
| A An instruction to remain silent during discomfort | B Communication of the concern and a dentist explanation of the plan and pause signal |
| C His own sedation prescription | D A guarantee of no sensation |

Answers and explanations: [page 93](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

dental anxiety

Apprehension or fear related to dental care.

IN USE acknowledge dental anxiety

distress

Significant discomfort or emotional upset.

IN USE notice reported distress

comfort measure

An appropriate action intended to support comfort during care.

IN USE discuss comfort measures

pain concern

A question or fear about pain that needs to be heard and addressed.

IN USE relay a pain concern

sensation

Something physically felt, such as pressure or discomfort.

IN USE describe a reported sensation

pressure

A physical sensation whose expected meaning requires the relevant clinical explanation.

IN USE clarify expected pressure

numbness

Reduced or absent sensation in a particular area.

IN USE report numbness accurately

local anesthesia

A clinical method of reducing sensation in a defined area.

IN USE discuss local anesthesia

topical anesthetic

An anesthetic applied to a surface for an appropriate clinical purpose.

IN USE clarify a topical anesthetic question

sedation

Medication-related reduction of awareness or anxiety at a defined level under clinical supervision.

IN USE discuss sedation options

analgesia

Relief of pain through the appropriate method or treatment.

IN USE clarify an analgesia question

contraindication

A reason a treatment may be inappropriate in a particular situation.

IN USE refer a contraindication question

Precision check

Pain, pressure, numbness, and anxiety are different words, but the assistant must not use that distinction to dismiss a patient report. The clinician should explain the actual comfort plan and expected sensations; the patient should not be promised an unqualified outcome.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

medical history

Relevant past and current health information.

IN USE update the medical history

previous experience

An earlier event affecting expectations or concerns.

IN USE acknowledge a previous experience

trigger

Something associated with an anxiety or distress response.

IN USE ask about a reported trigger

pause signal

An agreed way to request a pause during care.

IN USE agree a pause signal

ongoing consent

Consent addressed throughout the care process, not only at its beginning.

IN USE check ongoing consent

stop request

A request that the activity be stopped through the appropriate safe response.

IN USE acknowledge a stop request

explanation before action

Information given before the relevant step occurs.

IN USE request explanation before action

communication preference

The person's preferred way of receiving or giving information.

IN USE clarify a communication preference

reassurance

Information or support intended to reduce concern without misleading.

IN USE give honest reassurance

absolute guarantee

A promise with no stated uncertainty or exceptions.

IN USE avoid an unsupported absolute guarantee

clinician review

Assessment or explanation by the responsible qualified clinician.

IN USE request clinician review

shared understanding

Agreement about what has been explained and understood.

IN USE establish shared understanding

Precision check

Agreeing a pause signal is a communication step, not a universal gesture prescribed by this book. The clinician should confirm how to signal, how the team will respond safely, and how ongoing questions and consent will be addressed in the actual procedure.

Phrases you can actually use

C Notice the language / 1 of 2

ACKNOWLEDGE ANXIETY

"Thank you for telling me that you are worried."

AVOID A GUARANTEE

"I cannot promise that you will feel nothing."

PRESERVE THE WORDING

"I will tell the dentist you are worried about pain."

ASK ABOUT A SIGNAL

"Can we agree an appropriate way to request a pause?"

INVITE THE SPECIFIC CONCERN

"What would you like the dentist to know before starting?"

OFFER A USEFUL NEXT STEP

"I will ask the dentist to discuss the comfort plan with you."

REQUEST EXPLANATION

"Please explain the likely sensations and available comfort measures."

CONFIRM THE RESPONSE

"How will the team respond if you use that signal?"

Thank you for telling me

Makes reporting discomfort or anxiety welcome.

Cannot promise

Avoids certainty that the assistant cannot support.

Before starting

Places the discussion before the relevant action.

Phrases you can actually use

C Notice the language / 2 of 2

RESPECT A CONCERN

"Please tell us if something does not feel as explained."

CLARIFY PREFERENCE

"Would you prefer an explanation before each stage?"

SEPARATE REASSURANCE AND CERTAINTY

"I can help communicate your concern without promising an outcome."

RECORD ACCURATELY

"I will record your concern and the explanation actually provided."

AVOID MINIMIZATION

"I will not dismiss your concern as just nerves."

KEEP CLINICAL CHOICES WITH THE CLINICIAN

"The dentist needs to discuss any medication or sedation question."

SUPPORT QUESTIONS

"You can ask for clarification before deciding whether to continue."

CLOSE THE LOOP

"We need to confirm the plan and signal with you, not assume they are understood."

How will the team respond

Checks that a signal has an understood practical response.

As explained

Encourages reporting a difference without diagnosing it.

Actually provided

Keeps a pending explanation separate from a completed conversation.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which reassurance is appropriate?

- A** You will definitely feel nothing.
- B** Anxiety means your concern is not real.
- C** I will choose a sedative for you.
- D** I will help you discuss pain concerns and the comfort plan with the dentist before starting.

2. Which statement wrongly treats consent as permanent?

- A** You agreed once, so you cannot ask to stop.
- B** The clinician should discuss ongoing questions.
- C** The pause signal needs explanation.
- D** A changed concern should be communicated.

3. What should be confirmed about a pause signal?

- A** That it replaces all clinical discussion
- B** The actual signal and how the team will respond safely
- C** A universal gesture invented by the assistant
- D** That the patient must never use it

4. What should the record distinguish?

- A** Silence as proof that every question was answered
- B** Reported concern, explanation given, and any agreement actually reached
- C** A hoped-for painless outcome as a guaranteed fact
- D** A requested sedative as an administered drug

Answers and explanations: [page 93](#).

Reassure without promising a painless procedure

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

ongoing consent / comfort measure / communication preference / reassurance / absolute guarantee / Sedation / clinician review / sensation / dental anxiety / pause signal

- 01 **Ava:** Ben, I barely slept. Last time I felt something sharp and could not get anyone's attention. Can you promise this will not hurt?
- 02 **Ben:** Thank you for telling me about your ¹ _____. I cannot promise a painless experience, but I can make sure the dentist discusses your concern and the comfort plan before starting.
- 03 **Ava:** People say not to worry, but that makes me feel I should stay quiet if something bothers me.
- 04 **Ben:** I do not want ² _____ to silence you. Your concern needs a clear response, not a phrase that makes you feel responsible for hiding discomfort.
- 05 **Ava:** Last time I did not know whether I was supposed to feel pressure or pain. I could not tell what the person expected me to report.
- 06 **Ben:** We can ask about the expected ³ _____ in this procedure. I should not tell you which feeling is acceptable without the clinician's explanation or use the word pressure to dismiss what you report.
- 07 **Ava:** I would like to know what can be done to help me feel comfortable. I do not know which choices are appropriate for me.
- 08 **Ben:** That needs discussion of the actual ⁴ _____ or measures with the dentist. I will not invent a medication plan, but I will make sure the question is included.
- 09 **Ava:** Someone mentioned sedation to me, although I do not know whether they meant the same thing as having part of the mouth numb.
- 10 **Ben:** ⁵ _____ and local anesthesia are different terms. The dentist can explain their meanings and relevance to you; neither should be assumed simply because a patient has heard the word.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 46.

05

- 11 **Ava:** Please tell them about my previous experience too. I was not sure how to ask for a break once the person had started working.
- 12 **Ben:** We should ask the dentist to agree an appropriate ⁶ _____ with you before proceeding. The signal and the team's response need to be understood, rather than left for you to guess.
- 13 **Ava:** I would also like an explanation before each stage. Surprises make me tense, even when the action itself might not be difficult.
- 14 **Ben:** I can pass on that ⁷ _____. The team can discuss how to explain the sequence clearly and check what information helps you feel prepared.
- 15 **Ava:** If I agree to continue after the explanation and then change my mind, I want to know that I can still say something.
- 16 **Ben:** The clinician should address ⁸ _____ and respond to questions or a request to stop through the appropriate safe process. An earlier yes does not mean your later concern should be ignored.
- 17 **Ava:** That is more useful than a promise that nothing will hurt. I want an honest explanation and a way to communicate, not certainty nobody can give.
- 18 **Ben:** An unsupported ⁹ _____ would not help you make an informed choice. I can support a specific conversation about your concern, the plan, and how to ask for attention during care.
- 19 **Ava:** Could we discuss this before anything starts? I would feel less trapped with an agreed pause signal.
- 20 **Ben:** I will request that ¹⁰ _____ before we treat those questions as resolved. I will record what you asked and what is actually explained and agreed, not an assumed painless outcome.

ANSWERS / EXPLANATIONS: PAGE 94

1. dental anxiety | 2. reassurance | 3. sensation | 4. comfort measure | 5. Sedation | 6. pause signal | 7. communication preference | 8. ongoing consent | 9. absolute guarantee | 10. clinician review

Communicate a concern about unexpected sensations

F Say it

A patient requests explanation before each stage and asks how to request a pause. The clinician has not yet confirmed a signal. The assistant cannot guarantee particular sensations.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

guarantee / confirmed / each stage / pause signal

Assistant: "The requested communication preference is explanation before
¹ _____."

Patient: "I would like to agree a ² _____."

Assistant: "The signal is not yet ³ _____."

Assistant: "I cannot provide an absolute ⁴ _____ about sensations."

ANSWERS / EXPLANATIONS: PAGE 94

1. each stage | 2. pause signal | 3. confirmed | 4. guarantee

Rehearse with a fixed script

1. Read Ava and Ben aloud; make the request for a pause signal easy to hear.
2. Complete and read the transfer using its supplied concern, without dismissing pain as expected pressure.
3. Check the key, then repeat the request for the actual clinician explanation before treating the concern as resolved.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

A shared surname is not a verified match

A Read the situation

Report an identifier mismatch before using a record and hand off the facts without guessing which source is correct.

06

Assistant Diego is preparing a folder for a patient named Morgan. Two patients share that surname. The appointment list shows identifier ending 462, while the folder label shows 426. Diego has not used the folder or disclosed its contents. Lead assistant Priya can check the records through the approved identification process. Neither number is established as correct by this teaching brief. Diego must report the mismatch, preserve confidentiality, and avoid silently changing a label, merging records, or deciding that two similar numbers are merely a harmless typing error.

PEOPLE / Diego: Dental assistant | Priya: Lead assistant

SPEAK UP BEFORE A NEAR MATCH BECOMES A MATCH

A familiar surname and nearly identical digits can feel convincing during a busy clinic. State the exact mismatch without blaming the patient or a colleague. A neutral verification request is more useful than choosing whichever record looks most familiar.

Briefing checks

Circle one answer in each item. Use the briefing.

1. Which mismatch is supplied?

- | | |
|---|--|
| A The list ends 462 and the folder label ends 426. | B Both verified identifiers are identical. |
| C The surname is absent from both records. | D The appointment is confirmed for a different day. |

2. Which record is established as correct?

- | | |
|---|---------------------------------|
| A The folder automatically | B The list automatically |
| C Whichever was printed most recently without verification | D Neither |

3. What has Diego not done?

- | | |
|--|--|
| A Contacted a route for clarification in the scenario | B Used the folder or disclosed its contents |
| C Noticed the discrepancy | D Identified the shared surname |

Answers and explanations: page 95.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

patient identifier

Information used to distinguish a particular patient under the approved process.

IN USE verify patient identifiers

demographic detail

Recorded information such as name or date of birth.

IN USE verify demographic details

record number

An identifier assigned to a particular record.

IN USE confirm the record number

identifier mismatch

A disagreement between relevant identifying information.

IN USE report an identifier mismatch

name collision

Different people sharing the same or similar name in a system.

IN USE recognize a name collision

duplicate record

More than one record for the same person, requiring proper verification before consolidation.

IN USE investigate a possible duplicate record

wrong-patient risk

The possibility of using information or providing care for the wrong person.

IN USE address wrong-patient risk

chart

A patient's clinical record.

IN USE verify the chart

folder label

The identifying information shown on a physical folder.

IN USE check the folder label

appointment list

The schedule of booked patients and visits.

IN USE reconcile the appointment list

medical-history form

A record of relevant health information supplied and reviewed under the care process.

IN USE verify the medical-history form

odontogram

A diagrammatic dental chart of teeth and relevant recorded findings or treatment.

IN USE identify the odontogram

Precision check

A shared surname does not establish that two records concern the same patient. Similar digits do not prove a typing error, either. Preserve both values and use the full approved identification process; these three-digit endings are teaching references, not sufficient real-world identifiers.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

tooth notation

A system for identifying teeth in records.

IN USE confirm the tooth notation system

laterality

The specified side of the body or structure.

IN USE verify laterality

dentition

The arrangement or set of teeth present.

IN USE distinguish dentition references

chart entry

An item recorded in the patient chart.

IN USE make an accurate chart entry

source record

The original record used for verification or reference.

IN USE preserve the source record

reconciliation

Comparing records and resolving relevant differences through an approved process.

IN USE complete record reconciliation

traceable correction

An amendment that preserves who changed what and when.

IN USE make a traceable correction

audit trail

A history of recorded actions or changes.

IN USE preserve the audit trail

access control

Restriction of information access to authorized users and purposes.

IN USE follow access controls

confidential handoff

Transfer of relevant information through an appropriately protected route.

IN USE give a confidential handoff

unverified match

A possible link between person and record not yet confirmed.

IN USE flag an unverified match

record merge

Combining records after the required verification and authorization.

IN USE authorize a record merge

Precision check

Do not silently relabel or merge records to make the screen look consistent. Corrections require the appropriate verified basis and traceable process. A factual handoff can describe the mismatch and nonuse without claiming the investigation is already complete.

Phrases you can actually use

C Notice the language / 1 of 2

STATE THE DISCREPANCY

"The list and folder show different identifier endings."

NAME THE SHARED NAME

"Two patients in the records share this surname."

STATE THE CURRENT ACTION

"I have not used this folder."

REQUEST VERIFICATION

"Please check the match through the approved identification process."

READ THE VALUES CLEARLY

"The list ends four-six-two; the label ends four-two-six."

AVOID CHOOSING A WINNER

"I cannot yet establish which record is correct."

PROTECT INFORMATION

"I have not disclosed its contents."

AVOID A CASUAL CORRECTION

"I will not change the label just to make it match."

Different endings

Describes the observed mismatch without disclosing more information than necessary.

Four-six-two

Reading digits individually can make a transposition easier to detect.

Cannot yet establish

Avoids choosing a source without evidence.

Phrases you can actually use

C Notice the language / 2 of 2

KEEP SOURCES SEPARATE

"Let us preserve what each source currently shows."

CLARIFY A RECORD CHANGE

"Any correction needs the proper verified basis and audit trail."

SEPARATE IDENTIFICATION AND TREATMENT

"A record match does not itself confirm the treatment plan."

REPORT THE OUTCOME ACCURATELY

"I will record the actual verification result, not the expected result."

DO NOT ASSUME DUPLICATION

"A shared surname does not prove duplicate records."

PROTECT THE HANDOFF

"I will discuss the details through the approved confidential route."

CHECK NOTATION

"Which tooth notation system is used in this record?"

CLOSE ONLY WHEN RESOLVED

"The match remains unverified until the required checks are complete."

06

Have not used

Identifies the stage at which the issue was caught.

Verified basis

Requires evidence before altering records.

Actual result

Keeps investigation status separate from a hoped-for correction.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which report is precise?

- A** I corrected the label without checking so it looks consistent.
- B** The list ends 462, the folder ends 426, and I have not used the folder.
- C** The folder is definitely wrong because the list is on screen.
- D** Both records must belong to one person.

2. Why not merge the records immediately?

- A** Similar digits are proof of identity.
- B** Record merges never require verification.
- C** A shared surname does not establish that they concern the same person.
- D** Every name collision is a duplicate record.

3. What should a legitimate correction preserve?

- A** The verified basis and appropriate traceable history
- B** Only the new value with no history
- C** An invented earlier verification
- D** Another patient's details as a shortcut

4. What are the three-digit endings in this exercise?

- A** A universal minimum identification standard
- B** Proof of clinical consent
- C** Authorization to disclose the folder
- D** Teaching references, not a complete real-world identification method

Answers and explanations: [page 95](#).

A shared surname is not a verified match

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

06

WORD + PHRASE BANK

source record / odontogram / audit trail / identifier mismatch / patient identifier / name collision / wrong-patient risk / confidential handoff / record merge / traceable correction

- 01 **Diego:** Priya, I have an identifier mismatch before using this folder. The appointment list ends four-six-two, but the folder label ends four-two-six.
- 02 **Priya:** Keep the ¹ _____ unchanged while we check. Does the surname identify only one patient in our records, or is there another person with the same name?
- 03 **Diego:** There are two patients named Morgan. I have not used the folder or disclosed its contents, and I cannot establish which identifier belongs with this visit.
- 04 **Priya:** That is a ² _____, not proof that the records are duplicates. We need the approved identification process before choosing a chart or changing information.
- 05 **Diego:** Four-six-two and four-two-six look like swapped digits. I nearly trusted the screen over the folder, but I have no evidence that the screen is right.
- 06 **Priya:** An ³ _____ does not tell us which source is right. A plausible explanation still needs verification; the screen does not become authoritative merely because it is easier to read.
- 07 **Diego:** I will preserve the two values and the fact that the folder has not been used. That seems more helpful than deciding the cause in advance.
- 08 **Priya:** Yes. We should address the ⁴ _____ through the actual process, while keeping the description factual. We have identified a discrepancy before use, not established that treatment was given to the wrong person.
- 09 **Diego:** Should I ask the person a leading question with one of the numbers, or follow the usual identification questions without suggesting the answer?
- 10 **Priya:** Follow our approved ⁵ _____ checks. These partial numbers alone are not enough, and we should not improvise a shortcut that makes an uncertain match appear confirmed.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 55.

- 11 **Diego:** The folder also contains a dental chart, but I have not used its contents to guess whether it looks like the patient I remember.
- 12 **Priya:** Correct. An ⁶ _____ or a remembered treatment history should not replace the required identity checks. The relevant clinical record must first be linked to the correct person through the process.
- 13 **Diego:** If the review shows that one label is wrong, I should not remove the old information without recording how and why it was changed.
- 14 **Priya:** A ⁷ _____ is needed under the record procedure. The result must show the supported amendment, not pretend that the original discrepancy never existed.
- 15 **Diego:** And if the review reveals two records for one person, that would still be a separate authorized process rather than something I do because the surname matches.
- 16 **Priya:** Exactly. A ⁸ _____ needs the proper verification and authority. Joining records prematurely could mix information from different patients and make the problem harder to correct.
- 17 **Diego:** Can we check the details in the staff area? Both patients' names are visible here, and I do not want this conversation carrying into the waiting room.
- 18 **Priya:** That keeps this a ⁹ _____. We need enough information to resolve the issue, without exposing either patient's details to people who do not need them.
- 19 **Diego:** Until the check is complete, my update will say that the match is unverified, the folder has not been used, and you are coordinating the review.
- 20 **Priya:** Good. The ¹⁰ _____ should preserve the discrepancy, review, and actual resolution. We will close the issue on verified information rather than a near match that feels convincing.

06

ANSWERS / EXPLANATIONS: PAGE 96

1. source record | 2. name collision | 3. identifier mismatch | 4. wrong-patient risk | 5. patient identifier | 6. odontogram | 7. traceable correction | 8. record merge | 9. confidential handoff | 10. audit trail

Report a second identifier mismatch

F Say it

A schedule reference ends 815 and a folder reference ends 851. Two patients share a surname. The folder has not been used, and neither value is established as correct.

Complete the four gaps. Use each bank entry once.

06

WORD + PHRASE BANK

unverified / 851 / not been used / 815

Assistant: "The schedule reference ends ¹ _____."

Lead: "The folder reference ends ² _____."

Assistant: "The folder has ³ _____."

Lead: "The match remains ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 96

1. 815 | 2. 851 | 3. not been used | 4. unverified

Rehearse with a fixed script

1. Read the two endings four-six-two and four-two-six distinctly, then read the confidential handoff.
2. Complete the transfer with its different identifiers; do not choose a record simply because one value looks familiar.
3. Check the four answers, then repeat the distinction between a mismatch, an authorized correction, and a record merge.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Expected by noon is not ready for two

A Read the situation

Distinguish estimated delivery from clinical readiness and agree a useful laboratory update before confirming the patient plan.

Laboratory coordinator Felix estimates that case C62 will reach the practice on Tuesday by noon. The patient appointment is at 14:00 that day. Delivery is not guaranteed, and the dentist must confirm readiness before the appointment proceeds. Assistant Rina can obtain a laboratory status update on Monday at 16:00. She needs to separate production status, dispatch, receipt, and clinical readiness. The two-hour interval between estimated delivery and the appointment is not a guarantee of sufficient review time. No final decision to proceed or reschedule has been supplied.

PEOPLE / Rina: Dental assistant | Felix: Laboratory coordinator

07

AN ESTIMATE IS USEFUL WHEN ITS LIMITS TRAVEL WITH IT

Teams may shorten an expected delivery into it will be ready. Preserve the source, time, and conditions when passing information to the practice. A precise update request helps the patient plan without turning laboratory progress into a clinical promise.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What is the laboratory estimate?

- | | |
|--|---|
| A A completed clinical fit review | B Tuesday by noon, not guaranteed |
| C Monday at 16:00 delivery guaranteed | D Tuesday at 14:00 treatment confirmed |

2. When can the lab provide the next status update?

- | | |
|---|--|
| A A guaranteed arrival time of 16:00 | B Monday at 16:00 |
| C Tuesday at 14:00 only | D After the procedure is complete |

3. What else is required before proceeding?

- | | |
|---|---|
| A A two-hour gap alone | B Dentist confirmation of readiness |
| C Only the existence of tracking | D Automatic consent from the courier |

Answers and explanations: [page 97](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

laboratory case

A defined item of work handled by a dental laboratory.

IN USE track the laboratory case

case reference

The identifier used to distinguish a particular lab case.

IN USE verify the case reference

laboratory prescription

Authorized clinical instructions for the laboratory work.

IN USE verify the laboratory prescription

work authorization

Approval for the specified laboratory task.

IN USE confirm work authorization

impression

A negative reproduction used for making a model of oral structures.

IN USE identify the submitted impression

digital scan

A digital representation used within the relevant laboratory workflow.

IN USE verify the submitted digital scan

working model

A model used in producing the specified dental work.

IN USE identify the working model

shade

The specified color characteristics of a restoration or appliance.

IN USE confirm the shade reference

crown

A restoration covering the relevant tooth structure as clinically prescribed.

IN USE track a crown case

bridge

A restoration replacing missing teeth and supported as clinically designed.

IN USE identify the bridge case

denture

A prosthesis replacing missing teeth and related structures as prescribed.

IN USE identify the denture case

appliance

A device prescribed for a particular dental or orthodontic purpose.

IN USE verify the appliance type

Precision check

Produced, dispatched, delivered, checked, and ready for the appointment are different statuses. A tracking event does not necessarily confirm receipt of the correct case, and receipt does not replace the clinical readiness decision by the dentist.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

try-in

A clinical stage for assessing specified work before final completion or placement.

IN USE arrange a try-in appointment

fit assessment

Clinical assessment of whether the item fits as required.

IN USE obtain a fit assessment

occlusal check

Clinical assessment relating to how opposing teeth or restorations meet.

IN USE refer the occlusal check

remake

Production of replacement work after an appropriate reviewed request.

IN USE authorize a remake

production status

The current stage of laboratory manufacture.

IN USE request production status

dispatch status

Whether the item has actually been sent.

IN USE confirm dispatch status

tracking event

A recorded carrier event, not necessarily final receipt.

IN USE interpret a tracking event

estimated delivery

A forecast of when the item may arrive.

IN USE qualify estimated delivery

receipt confirmation

Verification that the specified item has arrived at the destination.

IN USE obtain receipt confirmation

clinical readiness

Confirmation that the relevant clinical requirements for proceeding are met.

IN USE verify clinical readiness

contingency booking

An alternative appointment arrangement considered under the practice process.

IN USE discuss a contingency booking

status checkpoint

An agreed time for reviewing and communicating progress.

IN USE set a status checkpoint

Precision check

Tuesday noon to 14:00 is a two-hour interval, but no sufficient review duration is established here. Keep the appointment decision with the practice and clinician. Use the Monday update to improve the information, not to guarantee a result before it exists.

Phrases you can actually use

C Notice the language / 1 of 2

IDENTIFY THE CASE

"I am checking case C62 for Tuesday's appointment."

REPEAT THE ESTIMATE

"You estimate arrival Tuesday by noon, without a guarantee."

NAME THE DEPENDENCY

"The dentist must confirm readiness before the appointment proceeds."

VERIFY THE SHIPMENT

"Please confirm the case reference associated with the tracking."

REQUEST THE STAGE

"What is the current production and dispatch status?"

SEPARATE THE CHECKPOINT

"Monday at sixteen hundred is the status update, not delivery."

AVOID A TIMING ASSUMPTION

"The two-hour gap does not itself establish enough review time."

DISTINGUISH DISPATCH AND RECEIPT

"A dispatch message does not confirm arrival at the practice."

07

Without a guarantee

Keeps the limitation attached to the forecast.

Before the appointment proceeds

States the separate clinical dependency.

Does not itself establish

Prevents a time calculation from becoming readiness evidence.

Phrases you can actually use

C Notice the language / 2 of 2

ASK ABOUT A CHANGE

"Please tell us if the estimate or production status changes."

CLARIFY A LAB QUESTION

"Any change to the clinical specification needs the proper authorization."

REQUEST A USEFUL UPDATE

"At the checkpoint, please distinguish finished, dispatched, and expected arrival."

CONFIRM THE HANDOFF

"I will pass the source and limits of the estimate to the dentist."

AVOID TREATMENT PROMISES

"I cannot tell the patient the appointment is ready based only on the estimate."

KEEP THE PATIENT PLAN SEPARATE

"The practice will decide the appointment response using the confirmed information."

PRESERVE UNCERTAINTY

"We have an estimate, not receipt confirmation."

CLOSE THE STATUS

"C62 remains subject to delivery and the required readiness confirmation."

Associated with the tracking

Checks that a real carrier event concerns the intended case.

Proper authorization

Keeps clinical specifications out of an improvised administrative change.

Source and limits

Helps the estimate survive a handoff without becoming a promise.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which patient update is accurate?

- A** The courier will decide clinical suitability.
- B** The lab estimates Tuesday noon delivery; the practice still needs receipt and dentist readiness confirmation.
- C** Tuesday treatment is guaranteed because a shipment is expected.
- D** Monday 16:00 is the confirmed delivery.

2. What is the interval from noon to 14:00?

- A** Two days
- B** A guaranteed clinical clearance period
- C** Two hours, with no proof that this is sufficient review time
- D** Four hours and automatic readiness

3. Which request is most useful at Monday's checkpoint?

- A** A silent change to the laboratory prescription
- B** Current production, actual dispatch, revised estimate, and any change affecting delivery
- C** A repeat of an old promise regardless of current facts
- D** A clinical diagnosis from the courier

4. What does a delivered status not establish by itself?

- A** A reason to check the case reference
- B** Clinical readiness to proceed
- C** That a carrier recorded a delivery event
- D** A need to verify receipt details

Answers and explanations: [page 97](#).

Expected by noon is not ready for two

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

clinical readiness / dispatch status / Receipt confirmation / production status / contingency booking / case reference / laboratory prescription / tracking event / estimated delivery / status checkpoint

07

- 01 **Rina:** Felix, I am checking case C62. The patient is booked for Tuesday at two, and I need to know exactly what your delivery estimate means.
- 02 **Felix:** The current ¹ _____ is Tuesday by noon, but it is not guaranteed. Please keep that qualification attached when you pass the timing to the practice.
- 03 **Rina:** That leaves two hours on paper. I am worried reception will hear Tuesday noon and tell the patient the two-o'clock fitting is definitely going ahead.
- 04 **Felix:** Correct. The ² _____ decision belongs with the dentist after the relevant checks. The laboratory estimate concerns arrival, not whether the appointment can definitely proceed.
- 05 **Rina:** What can you tell us at Monday's four-o'clock update? It would help to separate what is finished from what has actually left the laboratory.
- 06 **Felix:** We can report the ³ _____ and whether dispatch has occurred, along with the current delivery estimate. I will not call an item shipped simply because production is expected to finish.
- 07 **Rina:** If it has been sent, we also need the correct reference and tracking details. We sometimes receive more than one case in the same delivery.
- 08 **Felix:** We should verify the ⁴ _____ against the shipment. Tracking information for another item would not answer your question about C62, even if the destination is the same.
- 09 **Rina:** If the carrier website shows movement, I should describe that movement rather than say we have received the case in the practice.
- 10 **Felix:** Yes. A ⁵ _____ can show collection or transit without establishing receipt. Use the actual event and time instead of translating every update into arrived.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 64.

- 11 **Rina:** And when a package reaches us, the practice needs to confirm the right case was received and follow the relevant checks before calling it ready.
- 12 **Felix:** Exactly. ⁶ _____ and clinical readiness are separate. We should keep the sequence clear so a logistics message does not accidentally become a treatment promise.
- 13 **Rina:** If something changes in the clinical specification, I cannot simply amend it over this call to make the production schedule easier.
- 14 **Felix:** Any change to the ⁷ _____ needs the appropriate authorization and documentation. We can communicate a question, but the convenience of a delivery date does not decide clinical specifications.
- 15 **Rina:** The patient may need time to plan travel. I will ask the practice how to communicate the uncertainty and any alternative appointment arrangements.
- 16 **Felix:** A ⁸ _____ may be a practice decision, but I cannot make it from the laboratory. I can provide the clearest available facts and flag changes promptly.
- 17 **Rina:** Let me read those back: update Monday at four; estimated arrival Tuesday by noon. I will put them in separate fields so nobody confuses the dates.
- 18 **Felix:** That is the ⁹ _____. At that time we will distinguish production, dispatch, and expected arrival. If information changes earlier, we will communicate the change through the agreed route.
- 19 **Rina:** I will tell the dentist that no final decision to proceed or reschedule is established by this conversation. The appointment still depends on actual receipt and readiness.
- 20 **Felix:** Agreed. Our ¹⁰ _____ update will be factual and case-specific. C62 is expected Tuesday by noon, not guaranteed, and the dentist must make the relevant readiness decision.

07

ANSWERS / EXPLANATIONS: PAGE 98

1. estimated delivery | 2. clinical readiness | 3. production status | 4. case reference | 5. tracking event | 6. Receipt confirmation | 7. laboratory prescription | 8. contingency booking | 9. status checkpoint | 10. dispatch status

Separate a Wednesday delivery estimate from an update

F Say it

Case D14 is estimated for Wednesday at 10:00, without a guarantee. The appointment is at 13:00. The lab will update Tuesday at 15:00, and the dentist must still confirm readiness.

Complete the four gaps. Use each bank entry once.

07

WORD + PHRASE BANK

Tuesday at 15:00 / Wednesday at 10:00 / clinical readiness / D14

Assistant: "The case reference is ¹ _____."

Lab: "Estimated arrival is ² _____."

Assistant: "The status update is due ³ _____."

Lab: "The dentist must confirm ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 98

1. D14 | 2. Wednesday at 10:00 | 3. Tuesday at 15:00 | 4. clinical readiness

Rehearse with a fixed script

1. Read the C62 call with Monday at four and Tuesday by noon clearly separated.
2. Use the Wednesday transfer facts below to practice a different checkpoint and delivery estimate.
3. Check each blank, then repeat the final status without turning estimated delivery into clinical readiness.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

An unclear instruction needs an exact clinical answer

A Read the situation

Capture a postoperative question accurately, route it to the clinician, and distinguish routine clarification from urgent concerns.

Patient Owen calls assistant Farah because a phrase in his written postoperative instructions is unclear. He quotes "resume your usual routine as directed" and asks what that means for him. Farah is not authorized to reinterpret or change the instructions. The clinician can answer through the practice callback process, but no callback time or clinical answer is yet confirmed. Farah should preserve the exact wording and question, verify the contact through the approved process, and route it appropriately. An urgent or emergency concern must use the designated route rather than an ordinary queue.

PEOPLE / Owen: Patient | Farah: Dental assistant

08

CLARIFICATION IS NOT A NUISANCE

Patients may feel embarrassed to call about a phrase that seems simple to staff. Thank the caller for checking. Use exact wording and an accurate handoff so the clinician answers the real question rather than a summary that silently changes the instruction.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What does Owen need?

- A** Clarification of a phrase in his actual postoperative instructions
- C** Confirmation that every symptom is normal

- B** A new prescription invented by Farah
- D** A laboratory dispatch update

2. What is outside Farah's supplied role?

- A** Explaining that timing is not confirmed
- C** Recording the exact question

- B** Reinterpreting or changing the instructions
- D** Using the approved callback process

3. What overrides an ordinary callback queue?

- A** A wish to keep every call short
- C** An unconfirmed assumption that all callers are stable

- B** The existence of a printed sheet alone
- D** The appropriate route for an urgent or emergency concern

Answers and explanations: page 99.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

postoperative

Relating to the period after an operation or procedure.

IN USE clarify postoperative instructions

aftercare

Care and instructions relevant after treatment.

IN USE discuss an aftercare question

discharge instructions

The information given for care and follow-up after leaving the service.

IN USE verify discharge instructions

written direction

An instruction recorded in text whose actual wording matters.

IN USE quote the written direction

clinical clarification

An explanation from the appropriate clinician about the intended instruction.

IN USE request clinical clarification

verbatim

Using the actual words rather than a changed summary.

IN USE record the question verbatim

patient-reported concern

An issue described by the patient and attributed to them.

IN USE document a patient-reported concern

callback request

A request for a return call, not confirmation that the call occurred.

IN USE log a callback request

contact verification

Checking relevant contact details through the approved process.

IN USE complete contact verification

telephone note

A factual record of the call and actions taken.

IN USE complete a telephone note

triage route

The designated process for assessing and directing the concern.

IN USE use the triage route

urgent concern

A concern requiring timely action under the applicable clinical process.

IN USE escalate an urgent concern

Precision check

A familiar phrase can still have a patient-specific meaning. Do not silently replace as directed with an invented timetable or permission. Preserve the wording and have the clinician explain the intended direction and any relevant conditions.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

emergency response

The immediate appropriate response to an emergency under local arrangements.

IN USE use the emergency response

safety-net advice

Clinician-approved information on what to do if symptoms change or concerns arise.

IN USE clarify safety-net advice

review interval

The specified period before reassessment or follow-up.

IN USE confirm the review interval

follow-up appointment

A later visit arranged for a stated clinical purpose.

IN USE verify the follow-up appointment

extraction site

The area where a tooth has been removed.

IN USE identify an extraction-site question

suture

Material used to hold tissues together as part of a clinical procedure.

IN USE clarify a suture question

swelling

Enlargement of tissue whose meaning depends on clinical context.

IN USE report swelling accurately

bleeding

Loss of blood whose severity and context require appropriate assessment.

IN USE report bleeding without minimizing it

infection concern

A reported worry about possible infection, not itself a diagnosis.

IN USE route an infection concern

dry socket

A painful post-extraction complication requiring dental assessment; not a label for any discomfort.

IN USE refer a dry-socket question

instruction amendment

An authorized change to the actual clinical directions.

IN USE document an instruction amendment

unresolved question

A question for which an appropriate answer has not yet been provided.

IN USE track an unresolved question

Precision check

A callback request is not a completed answer. Record actual contact, instructions, and follow-up accurately. If the caller describes an urgent or emergency concern, use the appropriate local route promptly; the absence of such details in a language exercise is not a clinical clearance.

Phrases you can actually use

C Notice the language / 1 of 2

WELCOME THE QUESTION

"Thank you for calling to check the instruction."

PRESERVE THE SOURCE

"I will record that as wording from your instruction sheet."

STATE THE LIMIT

"I am not authorized to reinterpret or change that direction."

AVOID INVENTED TIMING

"I do not yet have a confirmed callback time."

ASK FOR EXACT WORDING

"Could you read the phrase exactly as it appears?"

CLARIFY THE QUESTION

"What part would you like the clinician to explain?"

OFFER THE ROUTE

"I can pass the exact question to the clinician through our callback process."

VERIFY CONTACT PROPERLY

"Let us confirm the contact details through our usual process."

08

Exactly as it appears

Preserves the wording that needs explanation.

What part

Identifies the actual uncertainty without guessing the intended question.

Not authorized to reinterpret

Separates communication support from clinical alteration.

Phrases you can actually use

C Notice the language / 2 of 2

KEEP URGENCY SEPARATE

"An urgent concern needs the designated route, not the ordinary queue."

RECORD AN ATTRIBUTED REPORT

"I will record what you tell me as your report."

CHECK THE RESPONSE

"Please ask for clarification if the clinician's answer remains unclear."

TRACK THE OPEN QUESTION

"The question remains unresolved until the appropriate answer is provided."

AVOID A DIAGNOSIS

"I cannot identify the cause of a symptom from an unassessed description."

DO NOT REPLACE THE INSTRUCTION

"I will not substitute my own timetable for the clinician's direction."

CONFIRM ANY AMENDMENT

"A changed instruction needs the authorized clinical response."

CLOSE ACCURATELY

"I have logged the request; I have not given a new clinical instruction."

Do not yet have

Avoids an unsupported promise of a specific response time.

Your report

Attributes symptoms or events to the caller rather than claiming direct observation.

Logged versus answered

Keeps the administrative action distinct from clinical resolution.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which response is appropriate?

- A** Any written instruction can be replaced by my personal advice.
- B** Calling proves no urgent concern exists.
- C** Please quote the phrase; I will route the exact question for clinical clarification without changing it.
- D** As directed always means immediately.

2. What should Farah say about callback timing?

- A** The clinician will call in five minutes without confirmation.
- B** The answer has already been given because the request was logged.
- C** Every callback may safely wait indefinitely.
- D** No time is confirmed yet.

3. What should happen if the caller reports an urgent concern?

- A** Diagnose it from a single word.
- B** Tell the caller that all postoperative symptoms are normal.
- C** Use the designated urgent or emergency route as appropriate.
- D** Leave it in the ordinary queue regardless.

4. Which note preserves the distinction?

- A** Exact phrase and question logged; clinician response pending
- B** New clinical advice supplied by Farah when none was authorized
- C** Concern resolved because a form was completed
- D** Original instruction silently replaced

Answers and explanations: page 99.

An unclear instruction needs an exact clinical answer

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

callback request / patient-reported concern / instruction amendment / contact verification / urgent concern / written direction / clinical clarification / verbatim / aftercare / telephone note

08

- 01 **Owen:** Farah, I am home with the instruction sheet, but one sentence has me stuck. Could someone explain it before I guess what I am supposed to do?
- 02 **Farah:** Thank you for checking the ¹ _____. Could you read the phrase exactly as it appears, then tell me which part you want the clinician to explain?
- 03 **Owen:** It says, resume your usual routine as directed. I do not know what as directed refers to or whether another part of the sheet explains it.
- 04 **Farah:** I will record that ² _____, along with your question. I am not authorized to reinterpret or change the instruction, so I should not supply a timetable that the clinician has not confirmed.
- 05 **Owen:** I am glad you said that. I do not need a general answer copied from someone else's visit; I need to know what the direction means for mine.
- 06 **Farah:** That requires ³ _____. I can pass the exact wording and your question through the practice callback process, using the relevant verified record and contact details.
- 07 **Owen:** Will the dentist actually see that wording? Last time I was told a message had been logged, but I still did not know what to do.
- 08 **Farah:** A ⁴ _____ is not an answer. I will keep the response pending until the actual clinical communication is recorded, rather than treat the administrative entry as resolution.
- 09 **Owen:** When should I expect the call? Please tell me if the time is not confirmed.
- 10 **Farah:** I do not have a confirmed time. We should complete ⁵ _____ through our process and use the designated route for an update on timing, rather than promise five minutes without confirmation.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 73.

- 11 **Owen:** If I report a new problem as well as the wording question, should that go into the same ordinary queue automatically?
- 12 **Farah:** No. An ⁶ _____ must use the designated route, including emergency response when appropriate. A call that begins with a routine question can still need a different response.
- 13 **Owen:** I understand. I do not want anyone to assume that every symptom after treatment is normal, or to guess a diagnosis from one word.
- 14 **Farah:** I will preserve any ⁷ _____ accurately and follow the practice process. I should not reassure you that a described symptom is harmless without the appropriate clinical assessment.
- 15 **Owen:** When the clinician responds, I want to be able to repeat what I understood and ask again if the explanation is still unclear.
- 16 **Farah:** That can help clarify the ⁸ _____ discussion. Ask about anything unclear in the actual response rather than rely on a guess simply because the original phrase sounded familiar.
- 17 **Owen:** And if the clinician changes the direction, I would like the new wording clear. I do not want two different instructions without knowing which applies.
- 18 **Farah:** Any ⁹ _____ needs to be communicated and recorded through the proper process. I will not create a competing instruction myself or silently replace the original text.
- 19 **Owen:** Please record the phrase and my question about its meaning. You have not given me a new clinical direction.
- 20 **Farah:** Correct. My ¹⁰ _____ will preserve the question, the action taken, and the pending response. We will use the appropriate route for any urgent concern rather than let the callback process delay it.

ANSWERS / EXPLANATIONS: PAGE 100

1. written direction | 2. verbatim | 3. clinical clarification | 4. callback request | 5. contact verification | 6. urgent concern | 7. patient-reported concern | 8. aftercare | 9. instruction amendment | 10. telephone note

Route an unclear follow-up interval

F Say it

A patient asks what review as arranged means on the instruction sheet. No appointment date is confirmed in the available record. The assistant cannot invent an interval; the clinician and scheduling team must clarify through the approved process.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

interval / review as arranged / appointment date / unresolved

08

Assistant: "The quoted phrase is ¹ _____."

Patient: "The missing information is the confirmed ² _____."

Assistant: "I cannot invent a review ³ _____."

Assistant: "The question is still ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 100

1. review as arranged | 2. appointment date | 3. interval | 4. unresolved

Rehearse with a fixed script

1. Read the exact aftercare wording before reading the request for clarification.
2. Complete the transfer using review as arranged and the missing appointment date, not an invented interval.
3. Check the key, then repeat the pending-response status and the distinction between a routine callback and an urgent concern.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Which tooth does twelve mean?

E More workplace conversations / 1-10

Read back the notation system, anatomical tooth, surfaces, and record status without inventing a treatment decision.

An imported note says 12 MOD without a notation-system label. The local chart uses Universal numbering. Dentist Imani confirms the intended tooth is the permanent upper-left first premolar, surfaces mesial, occlusal, distal, and the entry is planned, not completed. Assistant Jo verifies the charting instruction.

PEOPLE / Jo: Dental assistant | Dr. Imani: Dentist

Use each bank entry once, without changing its form. Answers follow turn 20.

09

WORD + PHRASE BANK

planned / distal / FDI / notation system / audit trail / Universal

- 01 **Jo:** Before I enter this, the imported note says twelve MOD. It does not name the numbering system. Which tooth did you intend?
- 02 **Dr. Imani:** The permanent upper-left first premolar. In our ¹ _____ chart that is number twelve. Please read back the tooth name as well as the number.
- 03 **Jo:** Permanent upper-left first premolar, Universal twelve. I nearly read the imported twelve as FDI one-two, which would be the upper-right lateral incisor.
- 04 **Dr. Imani:** Exactly. For the intended first premolar, the ² _____ equivalent is two-four. The conversion helps us check the language; it does not establish what an unlabeled source intended.
- 05 **Jo:** So I should preserve the imported wording and record your clarification, not silently replace every twelve in the source with twenty-four.
- 06 **Dr. Imani:** Correct. Keep the ³ _____ visible in the appropriate field. A correct conversion attached to the wrong source assumption would still be wrong.
- 07 **Jo:** Now the surfaces: I read MOD as mesial, occlusal, and distal. Is that the full entry, rather than MO?
- 08 **Dr. Imani:** Yes. Include ⁴ _____ as well as mesial and occlusal. Do not drop the final letter because the imported print is faint.
- 09 **Jo:** The screen offers existing, planned, and completed. Which status belongs with this entry? I do not want a treatment plan recorded as work already performed.
- 10 **Dr. Imani:** Use ⁵ _____. This instruction does not say that today's treatment has been carried out. Keep any existing restoration information separate from this proposed work.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 76.

- 11 **Jo:** I have the three parts: Universal twelve, MOD, planned. Does the charting clarification itself confirm consent to proceed today?
- 12 **Dr. Imani:** No. Confirming a record entry is not the whole clinical and consent process. We still follow those processes for the actual patient and proposed care.
- 13 **Jo:** I will not create a completed procedure entry or a charge from this clarification. I will enter only what you have actually confirmed within my authorized role.
- 14 **Dr. Imani:** Good. If the imported record needs an amendment, preserve the ⁶ _____. Do not make it look as though the original system label was present all along.
- 15 **Jo:** There is another twelve on the next page, but it has no tooth name. Should I apply today's clarification to that one too?
- 16 **Dr. Imani:** Not automatically. It may refer to another entry or a different context. Flag it separately so its source and intended tooth can be checked.
- 17 **Jo:** Understood. Today's confirmation covers this entry only. I will leave the second ambiguity visible rather than make the whole record appear reconciled.
- 18 **Dr. Imani:** That is right. Also keep left and right from the patient's perspective. A display facing us does not reverse the anatomical tooth name.
- 19 **Jo:** Final read-back: permanent upper-left first premolar, Universal twelve, equivalent FDI two-four; mesial, occlusal, distal; planned. Separate unlabeled entry still needs clarification.
- 20 **Dr. Imani:** Confirmed. Record this clarification with its source and date through our process. The other entry remains unresolved, and no completed treatment is established by this exchange.

09

ANSWERS / EXPLANATIONS: PAGE 101

1. Universal | 2. FDI | 3. notation system | 4. distal | 5. planned | 6. audit trail

Separate the number from the status

F Say it in a new situation

The dentist confirms the permanent upper-right lateral incisor: FDI 12, Universal 7. The entry describes an existing restoration, not a newly completed procedure. The imported notation must remain traceable; the assistant is authorized to record this clarification.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

existing / 7 / 12 / audit trail

09

Assistant: In FDI notation, the confirmed tooth is ¹ _____.

Dentist: Its Universal designation is ² _____.

Assistant: The confirmed record status is ³ _____.

Dentist: Preserve the source and clarification in the ⁴ _____.

ANSWERS / EXPLANATIONS: PAGE 101

1. 12 | 2. 7 | 3. existing | 4. audit trail

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

Update the medical history

E More workplace conversations / 1-10

Elicit and attribute a reported past reaction, distinguish unknown from none known, and obtain clinical review without diagnosing an allergy.

The chart says no known drug allergies. Patient Ravi now reports a past rash after an antibiotic but cannot recall the drug, year, or number of doses. There is no current reaction. Assistant Mei may collect history, but the dentist must assess the report before the visit proceeds.

PEOPLE / Mei: Dental assistant | Ravi: Patient

Use each bank entry once, without changing its form. Answers follow turn 20.

WORD + PHRASE BANK

no known drug allergies / information source / drug name / clinical review / unknown / reported reaction

10

- 01 **Mei:** Before we continue, has anything changed in your medical history or medicines since the last visit? That includes anything you forgot to mention before.
- 02 **Ravi:** I remembered a rash after an antibiotic years ago. The form says
1 _____, but I am not sure that still describes what I have told you.
- 03 **Mei:** Thank you for raising it. Are you describing a past event, or is anything happening now?
- 04 **Ravi:** A past event. I have no rash or other symptoms now. I cannot remember the
2 _____, though my old pharmacy might have a record.
- 05 **Mei:** I will record what you recall and what you do not. What did you notice at the time, and did anyone assess it?
- 06 **Ravi:** Red, itchy patches on my arms. I spoke to a doctor, but I cannot remember what they called it. That is my 3 _____, not a diagnosis I can confirm.
- 07 **Mei:** Do you remember the year, how long you had taken the medicine, or how soon the patches appeared after a dose?
- 08 **Ravi:** No. Please mark the timing as 4 _____. I do not want to guess a date just to finish the form.
- 09 **Mei:** That is helpful. Do you have a letter, medication list, or other record we could ask about through the proper process?
- 10 **Ravi:** Not with me. I can give the old pharmacy's details. My recollection is the current
5 _____, not something you have already verified with them.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 79.

- 11 **Mei:** Exactly. I will identify you as the source and keep the gaps visible. I will also tell the dentist that the old entry needs review before we proceed.
- 12 **Ravi:** Does that mean you are adding penicillin allergy? That is the antibiotic name people usually mention to me.
- 13 **Mei:** I cannot substitute that name for an unknown medicine or diagnose the cause. The dentist needs to assess your account and decide the appropriate next step.
- 14 **Ravi:** All right. I also started a supplement last month. Do you only need prescribed medicines for this history update?
- 15 **Mei:** Please include the medicines and supplements requested by our history process, including nonprescription products. Bring the names or packaging details rather than relying on a description such as a small white tablet.
- 16 **Ravi:** Should I stop taking my usual tablets until somebody has checked the list?
- 17 **Mei:** I cannot advise you to stop or change them. I will relay that question for
6 _____, together with the information you have given me.
- 18 **Ravi:** Please make sure the dentist knows I am asking about a past event, not saying I have a reaction here today.
- 19 **Mei:** I will. My handoff will separate the past rash, unknown antibiotic and timing, no current reaction reported, old chart entry, and your question about regular medicines.
- 20 **Ravi:** That is accurate. I can supply the pharmacy details now, but please do not say the history has been confirmed until the proper review has actually happened.

10

ANSWERS / EXPLANATIONS: PAGE 102

1. no known drug allergies | 2. drug name | 3. reported reaction | 4. unknown | 5. information source | 6. clinical review

Do not turn uncertainty into an allergy label

F Say it in a new situation

A patient reports nausea after a pain medicine two years ago; its name is unknown. No current symptoms are reported. The old chart says no known allergies. The assistant records the account and routes it to the clinician; no allergy diagnosis or advice to change medicines has been given.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

nausea / review / unknown / allergy

10

Assistant: The symptom the patient recalls is ¹ _____.

Patient: The medicine's name is ² _____.

Assistant: This is a reported reaction, not a confirmed ³ _____.

Patient: The advice and history update still need clinician ⁴ _____.

ANSWERS / EXPLANATIONS: PAGE 102

1. nausea | 2. unknown | 3. allergy | 4. review

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

Check the material, not the box

E More workplace conversations / 1-10

Separate product identity, shade, lot, expiry, and availability when reconciling restorative-material stock.

Fictional stock check on 10 October 2026: composite A2 lot C18 has 4 syringes, expiry 30 September; lot C27 has 6, expiry 31 December. Same product and shade. C27's storage and other required checks are verified. The reorder point is 8 usable syringes; target stock is 12, with none on order.

PEOPLE / Ella: Dental assistant | Sam: Stock lead

Use each bank entry once, without changing its form. Answers follow turn 20.

WORD + PHRASE BANK

lot number / six / instructions for use / usable stock / expiry date / first-expiry-first-out

11

- 01 **Ella:** The inventory screen shows ten A2 composite syringes, but four belong to a lot that expired on September thirtieth. Today is October tenth.
- 02 **Sam:** Separate physical count from ¹ _____. Put the expired four on the required hold and check the other lot's identity and status before counting it as available.
- 03 **Ella:** The six are the same product and shade A2, lot C27, expiry December thirty-first. Its storage and other required checks are verified in our record.
- 04 **Sam:** Then usable quantity is six. The ² _____ matters because two visually similar boxes can have different histories and expiry dates.
- 05 **Ella:** The reorder point is eight usable syringes. Six is below it, even though the shelf contains ten. Nothing is already on order.
- 06 **Sam:** Raise the order under our process to the target of twelve: ³ _____ additional syringes. Do not order only two; two would reach the trigger, not our stated target.
- 07 **Ella:** I will check the exact product, shade, pack size, and unit of purchase. Six syringes is not automatically six boxes.
- 08 **Sam:** Correct. Also use ⁴ _____ for eligible stock under our procedure. That means comparing expiry dates, not assuming the oldest delivery is always the first to expire.
- 09 **Ella:** The expired lot arrived later than C27. If I had followed delivery order alone, I would not have caught that difference.
- 10 **Sam:** That is why the ⁵ _____ needs its own check. We do not extend it by opening the box or moving the material to a different drawer.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 82.

- 11 **Ella:** There is also an A3 syringe on the shelf. Can I include it in the A2 available count because it is the same product range?
- 12 **Sam:** No. A different shade is not the requested item. Any clinical substitution needs the appropriate decision, not an inventory assumption made to avoid a shortage.
- 13 **Ella:** And the bonding agent is a separate stock item, even though it is used in the same restorative workflow. Its count cannot fill the composite shortage.
- 14 **Sam:** Exactly. Check each product's current ⁶ _____. Do not carry over another manufacturer's storage or handling directions because the packaging looks familiar.
- 15 **Ella:** For the expired four, I will record C18, quantity four, the printed expiry, and the hold location. The stock lead will arrange the authorized disposition.
- 16 **Sam:** Keep those units out of available inventory while preserving their traceability. A hold is not the same as a completed supplier return or documented disposal.
- 17 **Ella:** The order request will specify six additional A2 syringes, subject to approval. I will confirm the supplier's pack quantity before sending it.
- 18 **Sam:** And do not add that request to received stock. Approved, ordered, dispatched, and received quantities are different stages, even when we expect delivery soon.
- 19 **Ella:** Summary: ten physical, four expired on hold, six usable, six to replenish to twelve. A3 and bonding agent remain separate items.
- 20 **Sam:** Agreed. Update the counts and the hold record, then route the replenishment request. We have identified the requirement; we have not claimed that new stock has arrived.

11

ANSWERS / EXPLANATIONS: PAGE 103

1. usable stock | 2. lot number | 3. six | 4. first-expiry-first-out | 5. expiry date | 6. instructions for use

Count syringes rather than cartons

F Say it in a new situation

Fictional target: 15 usable syringes. There are 5 verified usable syringes and 3 expired units on hold; nothing is on order. The supplier sells cartons of 5 syringes. A purchase request is prepared but neither approved nor received.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

8 / 10 / received / 2

Assistant: Physical quantity is ¹ _____ syringes.

Lead: The replenishment requirement is ² _____ syringes.

Assistant: That quantity equals ³ _____ supplier cartons.

Lead: New stock has not yet been ⁴ _____ .

11

ANSWERS / EXPLANATIONS: PAGE 103

1. 8 | 2. 10 | 3. 2 | 4. received

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.

2. Switch roles. Read it again with the word bank covered.

3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

01 Briefing and language checks

Clarifying the appointment purpose

A Briefing checks / page 4

1. What does the schedule show?

A. A thirty-minute consultation

The schedule confirms a consultation and its duration, not a particular procedure.

2. What does Lena expect?

D. A filling today

Lena's expectation is a filling, which differs from the recorded appointment type.

3. Who can clarify the clinical plan?

B. The dentist before Lena decides whether to continue

The dentist is available to explain the plan; a booking entry does not supply a clinical decision.

D Language checks / page 9

1. Which response is accurate?

C. The record shows a consultation; the dentist can clarify the plan before you decide.

The response separates scheduling, clinical explanation, and the patient's decision.

2. Which statement overpromises?

B. Your insurance will definitely pay because the visit is booked.

A booking does not establish coverage, payment, or the final treatment charge.

3. What should be handed to the dentist?

D. The mismatch between the expected filling and recorded consultation, plus the patient's questions

The dentist needs the actual expectation and questions to clarify the visit without fabricated consent or diagnosis.

4. What does attendance establish by itself?

C. The patient came to the appointment, not agreement to every procedure

Attendance is an event and does not replace the required explanation and consent process.

01 Conversation answers

Clarifying the appointment purpose

E Dialogue gaps

Pages 10-11

- 1. patient expectation.** Turn 2. The patient expectation is a filling today, which must be acknowledged without treating it as a confirmed booking.
- 2. appointment type.** Turn 4. The appointment type identifies the scheduled consultation and does not automatically authorize or confirm treatment.
- 3. appointment duration.** Turn 6. Appointment duration is the time allocated in the schedule, not a clinical judgment about the tooth.
- 4. chief concern.** Turn 8. The chief concern is the patient's central question about the tooth and treatment options, not an assistant's diagnosis.
- 5. treatment estimate.** Turn 10. The treatment estimate must specify the proposed care and cost basis rather than remain an unexplained number.
- 6. coverage estimate.** Turn 12. A coverage estimate is provisional payment information and is not a guarantee of insurer payment or final patient cost.
- 7. Informed consent.** Turn 14. Informed consent requires the applicable explanation and agreement process; staying for a consultation is not blanket treatment permission.
- 8. booking discrepancy.** Turn 16. The booking discrepancy is the difference between the expected filling and recorded consultation, without assigning blame or demanding treatment.
- 9. treatment plan.** Turn 18. The treatment plan concerns the proposed care and sequence; the dentist must explain it rather than the assistant promise same-day treatment.
- 10. decision time.** Turn 20. Decision time gives the patient an opportunity to understand the explanation and ask questions before agreeing to the proposed next step.

F Transfer conversation

Page 12

1. review

Assistant: "The recorded appointment type is a review."

The schedule states a review rather than a confirmed fitting procedure.

2. crown fitting

Patient: "I expected a crown fitting."

Crown fitting is the patient's stated expectation, not the confirmed booking.

3. twenty

Assistant: "The allocated time is twenty minutes."

The supplied schedule allocates twenty minutes and does not guarantee a procedure.

4. unconfirmed

Assistant: "Fitting readiness remains unconfirmed."

The dentist has not confirmed readiness, so the assistant must preserve that status.

02 Briefing and language checks

Room readiness and safe clarification

A Briefing checks / page 13

1. What is known about P18?

C. It is present, but required readiness documentation cannot be confirmed.

The brief establishes an unresolved verification gap, not a confirmed failure or completed readiness.

2. What may Bruno provide?

B. A replacement whose readiness has been verified

A verified replacement may be available, but availability itself has not yet been confirmed.

3. What must not happen?

D. Treating pack presence as permission to skip the required checks

The assistant lacks authority to waive checks, and physical presence does not establish readiness.

D Language checks / page 18

1. Which message is best?

D. Please pause; P18's required readiness record is unconfirmed, and I need the approved review or a verified replacement.

The message identifies the item, gap, and appropriate response without guessing a result or waiving the process.

2. What does a color change alone establish here?

B. Not completion of every required readiness check

A single observation does not replace the complete applicable verification process.

3. What should happen to the original issue if a replacement is found?

D. It remains subject to the required review and record process.

A replacement supports the immediate workflow but does not establish or erase the original pack's history.

4. Which statement is precise?

C. Readiness is unverified; no confirmed failure is established in the supplied facts.

The statement preserves uncertainty while avoiding both a fabricated failure and an unsupported release.

02 Conversation answers

Room readiness and safe clarification

E Dialogue gaps

Pages 19-20

- 1. instrument pack.** Turn 2. The instrument pack identifier makes the concern specific and prevents confusion with another item or replacement.
- 2. release status.** Turn 4. Release status remains unconfirmed because the required checks have not been established; a busy schedule does not authorize use.
- 3. readiness documentation.** Turn 6. Readiness documentation supports the required verification; its absence is a gap, not by itself a confirmed contamination finding.
- 4. verified replacement.** Turn 8. A verified replacement must have its own readiness established; merely finding another package is insufficient.
- 5. Traceability.** Turn 10. Traceability connects the particular pack to its relevant processing history rather than accepting an unrelated record.
- 6. load number.** Turn 12. The load number identifies the relevant processing load; substituting another number would break the factual link.
- 7. chemical indicator.** Turn 14. A chemical indicator relates to specified process exposure and does not independently establish every readiness requirement.
- 8. hold.** Turn 16. The hold prevents premature use while the identified verification issue is being resolved through the required process.
- 9. cycle record.** Turn 18. The cycle record concerns the original processing history, which still needs the appropriate review even if a replacement is available.
- 10. IFU.** Turn 20. The instructions for use are the manufacturer's applicable directions and remain part of the required process, not an optional reference.

F Transfer conversation

Page 21

1. Q7

Assistant: "The affected pack is Q7."

Q7 is the specific item whose readiness remains unresolved.

2. not used

Lead: "Its use status is not used."

The brief explicitly states that the pack has not been used.

3. Eva

Assistant: "The review contact is Eva."

Eva is the lead assistant identified for the review.

4. verified

Lead: "The replacement is requested but not yet verified."

A replacement request does not establish the alternative item's readiness.

03 Briefing and language checks

Chairside requests and read-backs

A Briefing checks / page 22

1. Which word did Noor hear?

B. Tray

The brief says tray was audible while the distinguishing part was missed.

2. What are the two listed possibilities?

A. Instrument tray and impression tray

Both tray types appear on the setup list, making the incomplete request ambiguous.

3. What does the dentist clarify?

D. The instrument tray is wanted for the current setup.

The clarification resolves the current item while leaving ordinary practice procedures in place.

D Language checks / page 27

1. Which question best resolves the actual ambiguity?

B. Did you mean the instrument tray or the impression tray?

The question identifies the two relevant alternatives and requests the missing distinction.

2. Which read-back is complete for the clarified request?

A. The instrument tray for the current setup, correct?

The complete read-back preserves the distinguishing item and current context.

3. What should Noor do with unfamiliar local shorthand?

D. Ask for the full name and confirm the intended meaning.

Local shorthand can vary, so explicit clarification is more reliable than assumed meaning.

4. What does confirmation of the request establish?

C. Shared understanding of the requested item, not completion of all readiness checks

A communication check resolves the instruction while separate procedural requirements remain applicable.

03 Conversation answers

Chairside requests and read-backs

03

E Dialogue gaps

Pages 28-29

- 1. interruption.** Turn 2. The interruption masked part of the request; it does not justify choosing an item by guesswork.
- 2. read-back.** Turn 4. The read-back repeats the item and timing so the dentist can confirm the intended request.
- 3. distinguishing detail.** Turn 6. Naming HVE distinguishes the requested high-volume evacuator from the listed saliva ejector.
- 4. instrument tray.** Turn 8. Instrument tray is the requested item, rather than the separate impression tray on the list.
- 5. ambiguous instruction.** Turn 10. Small suction does not reliably identify the device or whether this handwritten request is current.
- 6. setup briefing.** Turn 12. The setup briefing is the appropriate point to reconcile the equipment list before relying on it.
- 7. chairside.** Turn 14. Chairside exchanges take place beside the dental chair, where short requests still need precise identifying details.
- 8. impression tray.** Turn 16. The impression tray is specifically excluded from this current request, not declared unnecessary for every appointment.
- 9. closed-loop request.** Turn 18. The receiver repeats the specific request and the requester confirms it, closing the communication loop.
- 10. handoff.** Turn 20. The handoff transfers the confirmed items and the still-unresolved note without concealing either status.

F Transfer conversation

Page 30

1. device name

Assistant: "I heard suction but missed the device name."

The missing information is the distinguishing device name, not the general word suction.

2. HVE

Dentist: "The requested device is HVE."

The dentist specifically clarifies HVE rather than the saliva ejector.

3. read-back

Assistant: "I will confirm it with a complete read-back."

Repeating the complete request lets the dentist confirm or correct the understanding.

4. required

Dentist: "The ordinary readiness checks remain required."

Clarifying the device does not waive separate readiness requirements.

04 Briefing and language checks

Imaging questions and authorization

04

A Briefing checks / page 31

1. What does Marco ask?

A. Whether the image proves that a tooth needs treatment

The question concerns interpretation and treatment significance of the image.

2. What is Suki not authorized to do here?

B. Interpret the image or recommend treatment

The fictional role expressly excludes image interpretation and treatment recommendation.

3. What clinical finding is established in the brief?

B. None

The brief supplies a patient question but no interpreted finding or diagnosis.

D Language checks / page 36

1. Which response is appropriate?

C. I cannot interpret it, but I will ask the dentist to explain whether it supports treatment and why.

The response preserves the treatment question and routes interpretation to the appropriate clinician.

2. What is wrong with labeling an unexplained feature a cavity?

C. No interpreted finding is supplied, and Suki lacks authorization to diagnose it.

The proposed label invents a clinical conclusion that neither the facts nor the assistant's role supports.

3. Which distinction is necessary?

A. Producing an image and interpreting it are different activities.

Tasks connected with imaging have different competencies and permissions, so they cannot be treated as interchangeable.

4. Which question should reach the dentist?

D. Does the image support treatment, and how does it relate to the examination and options?

The question seeks the actual clinical significance and options rather than an unsupported shortcut.

04 Conversation answers

Imaging questions and authorization

E Dialogue gaps

Pages 37-38

1. diagnostic significance. Turn 2. Diagnostic significance concerns what the image means for identifying a condition, which requires the appropriate clinical explanation.

2. radiograph. Turn 4. A radiograph is the X-ray image; displaying it does not establish authority to diagnose or explain every feature.

3. radiolucent. Turn 6. Radiolucent describes relative image appearance and is not, by itself, the name of a disease or treatment decision.

4. correlation. Turn 8. Correlation here means considering the image alongside other relevant clinical information, not treating one feature as sufficient proof.

5. image annotation. Turn 10. An image annotation is an added label or mark whose purpose and source need explanation rather than assumed diagnostic meaning.

6. artifact. Turn 12. An artifact is a feature introduced by a technical or other non-anatomical factor; its presence here has not been established.

7. authorized interpreter. Turn 14. The authorized interpreter is the appropriately qualified professional who can assess the image within the actual clinical and legal framework.

8. treatment indication. Turn 16. The treatment indication is the clinical reason for considering treatment, which must be explained rather than inferred from merely having an image.

9. comparison image. Turn 18. A comparison image is another image used for a defined clinical comparison; the scenario does not establish that one was reviewed.

10. image interpretation. Turn 20. Image interpretation is the professional assessment the patient is requesting; the assistant's handoff supports but does not supply it.

F Transfer conversation

Page 39

1. radiopaque

Assistant: "The term in your question is radiopaque."

Radiopaque is the supplied image-appearance term, not a confirmed treatment indication.

2. crown

Patient: "I want to know whether I need a crown."

The patient asks about a crown, but the question does not establish that it is needed.

3. dentist

Assistant: "The explanation needs the dentist."

The dentist is the identified clinician who can explain the note and its significance.

4. established

Assistant: "No treatment conclusion has been established here."

The brief supplies no interpreted finding or confirmed treatment decision.

05 Briefing and language checks

Patient comfort, questions, and pauses

A Briefing checks / page 40

1. What does Ava ask Ben to promise?

D. That the procedure will not hurt

The stated request is a guarantee about pain, not a confirmed comfort plan.

2. What has not happened yet?

A. The dentist's discussion of comfort measures and Ava's concerns

The brief says the clinical comfort discussion has not yet taken place.

3. What can Ben appropriately arrange?

B. Communication of the concern and a dentist explanation of the plan and pause signal

Ben can support the clinical conversation without inventing medication, sensations, or a treatment plan.

D Language checks / page 45

1. Which reassurance is appropriate?

D. I will help you discuss pain concerns and the comfort plan with the dentist before starting.

The response offers concrete support without a false sensation guarantee or unauthorized clinical decision.

2. Which statement wrongly treats consent as permanent?

A. You agreed once, so you cannot ask to stop.

An earlier agreement does not remove the need to address ongoing consent and a stop request.

3. What should be confirmed about a pause signal?

B. The actual signal and how the team will respond safely

A useful signal requires shared understanding of both the request and the team's response.

4. What should the record distinguish?

B. Reported concern, explanation given, and any agreement actually reached

Accurate recording separates concerns, communication, and actual decisions rather than assuming an outcome.

05 Conversation answers

Patient comfort, questions, and pauses

E Dialogue gaps

Pages 46-47

- 1. dental anxiety.** Turn 2. Dental anxiety identifies the patient's apprehension and should be acknowledged rather than dismissed or treated as a diagnosis of the tooth.
- 2. reassurance.** Turn 4. Reassurance should provide honest support and information, not pressure the patient to suppress a concern.
- 3. sensation.** Turn 6. Sensation refers to what the patient may feel; the actual clinician must explain it in relation to the planned procedure.
- 4. comfort measure.** Turn 8. A comfort measure must be appropriate to the patient's circumstances and explained by the responsible clinical team.
- 5. Sedation.** Turn 10. Sedation and local anesthesia are different clinical concepts; neither is confirmed or selected in this scenario.
- 6. pause signal.** Turn 12. A pause signal is an agreed communication method whose meaning and safe response must be confirmed for the actual care.
- 7. communication preference.** Turn 14. The communication preference is the patient's request for explanation before each stage, not a demand for a particular clinical treatment.
- 8. ongoing consent.** Turn 16. Ongoing consent requires attention throughout care; an earlier yes does not erase later questions or a request to stop.
- 9. absolute guarantee.** Turn 18. An absolute guarantee about pain would claim certainty the assistant does not have and could undermine honest communication.
- 10. clinician review.** Turn 20. Clinician review supplies the qualified discussion of the actual plan; the assistant's acknowledgment alone does not complete it.

F Transfer conversation

Page 48

1. each stage

Assistant: "The requested communication preference is explanation before each stage."

The patient specifically asks for explanations before each stage of care.

2. pause signal

Patient: "I would like to agree a pause signal."

The request concerns how to communicate a need for a pause.

3. confirmed

Assistant: "The signal is not yet confirmed."

The clinician has not yet agreed the signal or its response.

4. guarantee

Assistant: "I cannot provide an absolute guarantee about sensations."

The supplied role does not support an unqualified promise about what the patient will feel.

06 Briefing and language checks

Records, identifiers, and factual handoffs

A Briefing checks / page 49

1. Which mismatch is supplied?

A. The list ends 462 and the folder label ends 426.

The two supplied identifier endings differ despite the shared surname.

2. Which record is established as correct?

D. Neither

The brief does not establish the correct source, so the approved process must resolve the mismatch.

3. What has Diego not done?

B. Used the folder or disclosed its contents

The brief says the folder has not been used and its contents have not been disclosed.

D Language checks / page 54

1. Which report is precise?

B. The list ends 462, the folder ends 426, and I have not used the folder.

The precise report preserves the values and action status without assigning unsupported correctness.

2. Why not merge the records immediately?

C. A shared surname does not establish that they concern the same person.

Different people can share a surname, so consolidation without verification could combine their information incorrectly.

3. What should a legitimate correction preserve?

A. The verified basis and appropriate traceable history

A traceable correction records the supported change rather than concealing how the discrepancy was resolved.

4. What are the three-digit endings in this exercise?

D. Teaching references, not a complete real-world identification method

The exercise uses shortened references for language practice and does not replace an approved full identification process.

06 Conversation answers

Records, identifiers, and factual handoffs

E Dialogue gaps

Pages 55-56

- 1. source record.** Turn 2. The source record preserves the original information needed to investigate the mismatch rather than replacing it with a guess.
- 2. name collision.** Turn 4. A name collision means more than one person shares a name; it does not prove the records should be combined.
- 3. identifier mismatch.** Turn 6. An identifier mismatch establishes disagreement between sources, not which source is correct or why the difference occurred.
- 4. wrong-patient risk.** Turn 8. Wrong-patient risk arises when an unverified record could be used for the wrong person; raising the mismatch before use supports prevention.
- 5. patient identifier.** Turn 10. Patient identifiers must be verified through the actual approved process, not by treating a prompted guess as reliable confirmation.
- 6. odontogram.** Turn 12. An odontogram records dental information and must not become an informal substitute for the required patient-identification checks.
- 7. traceable correction.** Turn 14. A traceable correction preserves the verified basis and history of a change rather than silently overwriting the discrepancy.
- 8. record merge.** Turn 16. A record merge combines information and therefore requires verified identity and the appropriate authority, not a shared surname alone.
- 9. confidential handoff.** Turn 18. A confidential handoff shares the relevant information through an appropriately protected route instead of exposing it to unrelated people.
- 10. audit trail.** Turn 20. The audit trail records relevant actions and changes so the actual review and correction history can be understood later.

F Transfer conversation

Page 57

1. 815

Assistant: "The schedule reference ends 815."

The supplied schedule value is 815, distinct from the folder value.

2. 851

Lead: "The folder reference ends 851."

The supplied folder value is 851 and must not be silently changed.

3. not been used

Assistant: "The folder has not been used."

The brief places the discrepancy before any use of the folder.

4. unverified

Lead: "The match remains unverified."

Neither source is established as correct, so the identification process remains necessary.

07 Briefing and language checks

Laboratory cases and delivery expectations

A Briefing checks / page 58

1. What is the laboratory estimate?

B. Tuesday by noon, not guaranteed

The lab estimates arrival by Tuesday noon and explicitly does not guarantee it.

2. When can the lab provide the next status update?

B. Monday at 16:00

Monday 16:00 is the update time, not the estimated delivery time.

3. What else is required before proceeding?

B. Dentist confirmation of readiness

The scenario separately requires the dentist to confirm readiness after the relevant checks.

D Language checks / page 63

1. Which patient update is accurate?

B. The lab estimates Tuesday noon delivery; the practice still needs receipt and dentist readiness confirmation.

The accurate message separates estimated arrival from the practice's required readiness decision.

2. What is the interval from noon to 14:00?

C. Two hours, with no proof that this is sufficient review time

The arithmetic gives two hours, but the brief supplies no rule making that sufficient.

3. Which request is most useful at Monday's checkpoint?

B. Current production, actual dispatch, revised estimate, and any change affecting delivery

The requested status gives the practice actionable facts without replacing clinical authority or current evidence.

4. What does a delivered status not establish by itself?

B. Clinical readiness to proceed

A delivery event concerns logistics and does not supply the dentist's separate readiness confirmation.

07 Conversation answers

Laboratory cases and delivery expectations

E Dialogue gaps

Pages 64-65

- 1. estimated delivery.** Turn 2. Estimated delivery is a forecast of Tuesday noon arrival, not a guaranteed appointment or confirmed receipt.
- 2. clinical readiness.** Turn 4. Clinical readiness requires the dentist's relevant checks and decision; a forecast delivery time does not establish it.
- 3. production status.** Turn 6. Production status identifies the manufacturing stage and must remain distinct from actual dispatch or delivery.
- 4. case reference.** Turn 8. The case reference links the status and tracking to C62 rather than another case in the same delivery.
- 5. tracking event.** Turn 10. A tracking event records a carrier action and is not necessarily confirmation that the intended case was received by the practice.
- 6. Receipt confirmation.** Turn 12. Receipt confirmation verifies arrival of the specified item; it remains separate from the clinical assessment needed before proceeding.
- 7. laboratory prescription.** Turn 14. The laboratory prescription contains authorized clinical instructions and cannot be altered casually to accommodate a schedule.
- 8. contingency booking.** Turn 16. A contingency booking is an alternative appointment arrangement for the practice to consider, not an automatic cancellation or confirmed replacement.
- 9. status checkpoint.** Turn 18. The status checkpoint is the agreed Monday update time and must not be confused with the Tuesday delivery estimate.
- 10. dispatch status.** Turn 20. Dispatch status records whether the case has actually been sent; it is one logistical fact, not final appointment approval.

F Transfer conversation

Page 66

1. D14

Assistant: "The case reference is D14."

D14 identifies the particular laboratory case in this exchange.

2. Wednesday at 10:00

Lab: "Estimated arrival is Wednesday at 10:00."

The supplied estimate concerns Wednesday arrival and is explicitly not guaranteed.

3. Tuesday at 15:00

Assistant: "The status update is due Tuesday at 15:00."

Tuesday 15:00 is the information checkpoint, not the delivery time.

4. clinical readiness

Lab: "The dentist must confirm clinical readiness."

Delivery timing alone does not establish readiness for the appointment.

08 Briefing and language checks

Aftercare questions and follow-up calls

A Briefing checks / page 67

1. What does Owen need?

A. Clarification of a phrase in his actual postoperative instructions

The caller asks what a particular written phrase means in his circumstances.

2. What is outside Farah's supplied role?

B. Reinterpreting or changing the instructions

The brief explicitly excludes independent reinterpretation and alteration of clinical instructions.

3. What overrides an ordinary callback queue?

D. The appropriate route for an urgent or emergency concern

Urgent concerns require their designated response rather than delay for routine clarification.

D Language checks / page 72

1. Which response is appropriate?

C. Please quote the phrase; I will route the exact question for clinical clarification without changing it.

The response preserves the question and routes it within the assistant's role.

2. What should Farah say about callback timing?

D. No time is confirmed yet.

The brief supplies no confirmed callback time, so neither a specific promise nor an indefinite safety claim is justified.

3. What should happen if the caller reports an urgent concern?

C. Use the designated urgent or emergency route as appropriate.

The appropriate urgent route takes precedence over routine clarification and avoids unsupported reassurance.

4. Which note preserves the distinction?

A. Exact phrase and question logged; clinician response pending

A factual note separates the request, action, and pending response without inventing clinical resolution.

08 Conversation answers

Aftercare questions and follow-up calls

E Dialogue gaps

Pages 73-74

1. written direction. Turn 2. The written direction is the actual text needing explanation, so its wording should be preserved rather than guessed.

2. verbatim. Turn 4. Verbatim recording preserves the actual phrase so the clinician can answer the real wording instead of a changed summary.

3. clinical clarification. Turn 6. Clinical clarification provides the appropriate patient-specific explanation of the instruction rather than an assistant's improvised interpretation.

4. callback request. Turn 8. A callback request records the need for contact but does not establish that the clinician has called or answered.

5. contact verification. Turn 10. Contact verification checks the appropriate details through the approved process so the response is routed correctly without assumptions.

6. urgent concern. Turn 12. An urgent concern requires its appropriate timely response and must not be downgraded merely because the call began as a wording question.

7. patient-reported concern. Turn 14. A patient-reported concern is attributed to the caller and routed for assessment rather than labeled normal or diagnosed without support.

8. aftercare. Turn 16. Aftercare concerns the actual post-treatment directions and follow-up, which should be understood rather than merely delivered.

9. instruction amendment. Turn 18. An instruction amendment is an authorized change to the directions and must be communicated and recorded clearly through the required process.

10. telephone note. Turn 20. The telephone note records the actual phrase, question, contact action, and pending response without claiming a clinical answer has been given.

F Transfer conversation

Page 75

1. review as arranged

Assistant: "The quoted phrase is review as arranged."

The exact phrase identifies the instruction that needs clarification.

2. appointment date

Patient: "The missing information is the confirmed appointment date."

No appointment date is confirmed in the available record.

3. interval

Assistant: "I cannot invent a review interval."

The assistant lacks authority to create a clinical follow-up timetable.

4. unresolved

Assistant: "The question is still unresolved."

Routing the question does not establish that an appropriate answer has already been provided.

09 Conversation answers

Which tooth does twelve mean?

E Extended conversation

Pages 76-77

1. Universal. Turn 2. Universal number twelve identifies the permanent upper-left first premolar; the identical digits in FDI notation identify a different tooth.

2. FDI. Turn 4. FDI two-four corresponds to the intended permanent upper-left first premolar; FDI one-two is not the same tooth as Universal twelve.

3. notation system. Turn 6. The notation system must accompany the tooth designation so a reader knows how to interpret the digits.

4. distal. Turn 8. The confirmed MOD entry includes distal as well as mesial and occlusal; recording MO would omit one of the specified surfaces.

5. planned. Turn 10. The dentist explicitly confirms planned status; the dialogue supplies no completed procedure to document or bill as completed.

6. audit trail. Turn 14. An audit trail preserves the source wording and the dated, attributable clarification instead of concealing the original ambiguity.

F Transfer exchange

Page 78

1. 12

Assistant: In FDI notation, the confirmed tooth is 12 .

FDI twelve identifies the permanent upper-right lateral incisor specified by the dentist.

2. 7

Dentist: Its Universal designation is 7 .

Universal seven identifies that same permanent upper-right lateral incisor, not Universal twelve.

3. existing

Assistant: The confirmed record status is existing .

The supplied instruction describes an existing restoration rather than work newly completed at this visit.

4. audit trail

Dentist: Preserve the source and clarification in the audit trail .

The correction must remain attributable and traceable rather than silently erasing the imported notation.

Background reading: [ADA: Universal Tooth Designation System and ISO equivalents](#). The conversation is an original fictional teaching case.

10 Conversation answers

Update the medical history

E Extended conversation

Pages 79-80

1. no known drug allergies. Turn 2. The existing no-known-allergies entry conflicts with a newly reported possible drug reaction and needs review; it should not be carried forward unquestioned.

2. drug name. Turn 4. The drug name is unknown in the supplied account; choosing a familiar antibiotic would invent information rather than clarify the history.

3. reported reaction. Turn 6. The reported reaction is Ravi's account of red, itchy patches, not a confirmed allergy diagnosis or a clinician's documented conclusion.

4. unknown. Turn 8. Unknown timing is a missing fact; it must not be converted into an immediate reaction or a statement that no reaction occurred.

5. information source. Turn 10. The information source is the patient's recollection until another record is actually obtained and checked; a possible pharmacy record is not yet corroborating evidence.

6. clinical review. Turn 17. Clinical review is needed for assessment and advice; collecting the history does not authorize the assistant to change medicines or clear the patient for treatment.

F Transfer exchange

Page 81

1. nausea

Assistant: The symptom the patient recalls is nausea .

Nausea is the supplied symptom; replacing it with rash would change the reported history.

2. unknown

Patient: The medicine's name is unknown .

The patient cannot identify the medicine, so the assistant must not invent a drug name.

3. allergy

Assistant: This is a reported reaction, not a confirmed allergy .

The facts provide an account requiring assessment, not a clinical conclusion that the reaction was allergic.

4. review

Patient: The advice and history update still need clinician review .

The report has been routed but not clinically assessed or resolved in the supplied exchange.

10

Background reading: [NICE CG183: documenting and sharing suspected drug-allergy information](#). The conversation is an original fictional teaching case.

11 Conversation answers

Check the material, not the box

E Extended conversation

Pages 82-83

1. usable stock. Turn 2. Only the six verified, in-date syringes count as usable stock; the four expired syringes remain physically present but unavailable for use.

2. lot number. Turn 4. The lot number distinguishes C27 from expired C18 even though their product and shade are the same.

3. six. Turn 6. Twelve target syringes minus six usable syringes leaves a six-syringe replenishment requirement, with no outstanding order to subtract.

4. first-expiry-first-out. Turn 8. First-expiry-first-out prioritizes the earliest expiry among eligible stock; it does not authorize using already expired material.

5. expiry date. Turn 10. The expiry date is a separate label field from receipt date and lot number; opening or moving a pack does not extend it.

6. instructions for use. Turn 14. The instructions for use are product-specific; another material's storage, handling, or curing directions cannot be applied merely because the products are used together.

F Transfer exchange

Page 84

1. 8

Assistant: Physical quantity is 8 syringes.

Five usable plus three expired units equals eight physically present, but not eight usable.

2. 10

Lead: The replenishment requirement is 10 syringes.

The target of fifteen less five usable syringes requires ten additional syringes.

3. 2

Assistant: That quantity equals 2 supplier cartons.

Ten required syringes divided by five per carton equals two cartons.

4. received

Lead: New stock has not yet been received .

Preparing a purchase request does not establish approval, shipment, or physical receipt of supplies.

11

Background reading: [GC: composite product identification, shades, and manufacturer instructions](#). The conversation is an original fictional teaching case.

Find the move you need

Use these as precise language models. Replace details only when you know the actual facts, authority, and agreed conditions.

01 Clarifying the appointment purpose

Acknowledge the expectation: I understand you expected a filling today.

Avoid blame: Let us clarify the appointment before making assumptions.

Respect the decision: You can ask questions before deciding whether to continue.

Full phrase set: pages 7-8 | Dialogue: pages 10-11

02 Room readiness and safe clarification

Interrupt clearly: Please pause; I cannot confirm the required readiness check for P18.

Avoid an unsupported finding: I am not reporting a confirmed contamination event.

Ask about an alternative: Is a verified replacement available?

Full phrase set: pages 16-17 | Dialogue: pages 19-20

03 Chairside requests and read-backs

Signal the gap: I heard tray, but I missed the word before it.

Avoid guessing: I have not selected either yet.

Clarify sequence: Do you need that now or at the next stage?

Full phrase set: pages 25-26 | Dialogue: pages 28-29

04 Imaging questions and authorization

Acknowledge the question: You want to know whether the image means this tooth needs treatment.

Avoid a visual guess: I cannot identify a diagnosis from that feature for you.

Ask for ordinary language: Could the dentist explain that term in everyday language?

Full phrase set: pages 34-35 | Dialogue: pages 37-38

Keep the conversation productive

Use these as precise language models. Replace details only when you know the actual facts, authority, and agreed conditions.

05 Patient comfort, questions, and pauses

Acknowledge anxiety: Thank you for telling me that you are worried.

Offer a useful next step: I will ask the dentist to discuss the comfort plan with you.

Confirm the response: How will the team respond if you use that signal?

Full phrase set: pages 43-44 | Dialogue: pages 46-47

06 Records, identifiers, and factual handoffs

State the discrepancy: The list and folder show different identifier endings.

Avoid choosing a winner: I cannot yet establish which record is correct.

Avoid a casual correction: I will not change the label just to make it match.

Full phrase set: pages 52-53 | Dialogue: pages 55-56

07 Laboratory cases and delivery expectations

Identify the case: I am checking case C62 for Tuesday's appointment.

Separate the checkpoint: Monday at sixteen hundred is the status update, not delivery.

Distinguish dispatch and receipt: A dispatch message does not confirm arrival at the practice.

Full phrase set: pages 61-62 | Dialogue: pages 64-65

08 Aftercare questions and follow-up calls

Welcome the question: Thank you for calling to check the instruction.

Clarify the question: What part would you like the clinician to explain?

Verify contact properly: Let us confirm the contact details through our usual process.

Full phrase set: pages 70-71 | Dialogue: pages 73-74

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

absolute guarantee	avoid an unsupported absolute guarantee	42
access control	follow access controls	51
aftercare	discuss an aftercare question	68
air-water syringe	identify the air-water syringe	24
ambiguous instruction	clarify an ambiguous instruction	24
analgesia	clarify an analgesia question	41
anatomical structure	identify the named anatomical structure	33
appliance	verify the appliance type	59
appointment duration	confirm appointment duration	6
appointment list	reconcile the appointment list	50
appointment type	verify the appointment type	5
artifact	report a possible artifact	33
audit trail	preserve the audit trail	51
authorized interpreter	refer to the authorized interpreter	33
biological indicator	review biological indicator results	14
bitewing	identify the bitewing view	32
bleeding	report bleeding without minimizing it	69
booking discrepancy	resolve a booking discrepancy	6
bridge	identify the bridge case	59
bur	verify the bur identifier	24
callback request	log a callback request	68
caries	discuss diagnosed caries	6
case reference	verify the case reference	59
CBCT	identify the CBCT study	32

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

chairside provide chairside support	23
chart verify the chart	50
chart entry make an accurate chart entry	51
chemical indicator inspect the chemical indicator	14
chief concern clarify the chief concern	5
cleaning verify the cleaning step	14
clinical clarification request clinical clarification	68
clinical decision refer a clinical decision	6
clinical examination arrange a clinical examination	5
clinical finding explain a clinical finding	33
clinical readiness verify clinical readiness	60
clinician review request clinician review	42
closed-loop request complete a closed-loop request	24
comfort measure discuss comfort measures	41
communication preference clarify a communication preference	42
comparison image identify the comparison image	33
compromised packaging report compromised packaging	15
confidential handoff give a confidential handoff	51
consultation attend a consultation	5
contact verification complete contact verification	68
contingency booking discuss a contingency booking	60
contraindication refer a contraindication question	41
correlation correlate clinical information	33
cotton pliers identify the cotton pliers	23

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

coverage estimate qualify a coverage estimate	6
crown track a crown case	59
curing light identify the curing light	24
cycle record trace the cycle record	14
decision time allow decision time	6
decontamination follow decontamination procedures	14
definitive treatment discuss definitive treatment	5
demographic detail verify demographic details	50
dental anxiety acknowledge dental anxiety	41
dentition distinguish dentition references	51
denture identify the denture case	59
diagnostic significance ask about diagnostic significance	33
digital scan verify the submitted digital scan	59
discharge instructions verify discharge instructions	68
disinfection distinguish disinfection from sterilization	14
dispatch status confirm dispatch status	60
distinguishing detail request the distinguishing detail	24
distress notice reported distress	41
dry socket refer a dry-socket question	69
duplicate record investigate a possible duplicate record	50
emergency response use the emergency response	69
estimated delivery qualify estimated delivery	60
explanation before action request explanation before action	42
explorer identify the explorer	23

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

exposure document the exposure	32
extraction site identify an extraction-site question	69
extraoral image identify an extraoral image	32
field of view clarify the field of view	32
filling schedule a filling	5
fit assessment obtain a fit assessment	60
folder label check the folder label	50
follow-up appointment verify the follow-up appointment	69
handpiece clarify the handpiece type	24
hold maintain the hold	15
HVE clarify the HVE request	23
identifier mismatch report an identifier mismatch	50
IFU consult the IFU	15
image annotation clarify an image annotation	33
image interpretation refer image interpretation	33
image quality assess image quality within role	32
image receptor identify the image receptor	32
impression distinguish an impression from a scan	23
impression identify the submitted impression	59
impression tray identify the impression tray	23
infection concern route an infection concern	69
infection prevention follow infection prevention procedures	15
informed consent support informed consent	5
instruction amendment document an instruction amendment	69

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

instrument pack identify the instrument pack	14
instrument tray confirm the instrument tray	23
interruption acknowledge an interruption	24
intraoral image identify an intraoral image	32
intraoral scan identify the intraoral scan	23
laboratory case track the laboratory case	59
laboratory prescription verify the laboratory prescription	59
laterality verify laterality	51
load number verify the load number	14
local anesthesia discuss local anesthesia	41
matrix band identify the matrix band	24
mechanical monitoring review mechanical monitoring	14
medical history update the medical history	42
medical-history form verify the medical-history form	50
mirror identify the requested mirror	23
name collision recognize a name collision	50
numbness report numbness accurately	41
occlusal check refer the occlusal check	60
odontogram identify the odontogram	50
ongoing consent check ongoing consent	42
pack integrity check pack integrity	15
pain concern relay a pain concern	41
panoramic image identify a panoramic image	32
patient expectation acknowledge a patient expectation	6

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

patient identifier	verify patient identifiers	50
patient-reported concern	document a patient-reported concern	68
pause signal	agree a pause signal	42
periapical image	identify a periapical image	32
periodontal probe	distinguish the periodontal probe	23
postoperative	clarify postoperative instructions	68
presenting complaint	record the presenting complaint	5
pressure	clarify expected pressure	41
previous experience	acknowledge a previous experience	42
procedure code	verify the procedure code	6
processing deviation	report a processing deviation	15
production status	request production status	60
provisional recommendation	qualify a provisional recommendation	5
radiograph	identify the radiograph	32
radiography	distinguish radiography from interpretation	32
radiolucent	recognize the term radiolucent	33
radiopaque	recognize the term radiopaque	33
read-back	give a complete read-back	24
readiness documentation	confirm readiness documentation	15
reassurance	give honest reassurance	42
receipt confirmation	obtain receipt confirmation	60
reconciliation	complete record reconciliation	51
record merge	authorize a record merge	51
record number	confirm the record number	50

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

referral confirm the referral	6
release status confirm release status	15
remake authorize a remake	60
reprocessing follow reprocessing instructions	14
restoration discuss a restoration	5
restorative dentistry explain restorative dentistry	6
retractor identify the retractor	24
reusable device identify a reusable device	15
review appointment arrange a review appointment	6
review interval confirm the review interval	69
safety-net advice clarify safety-net advice	69
saliva ejector distinguish the saliva ejector	23
sedation discuss sedation options	41
sensation describe a reported sensation	41
setup briefing conduct a setup briefing	23
shade confirm the shade reference	59
shared understanding establish shared understanding	42
single-use device identify a single-use device	15
source record preserve the source record	51
status checkpoint set a status checkpoint	60
sterilization verify sterilization records	14
sterilizer identify the sterilizer	14
stop request acknowledge a stop request	42
suture clarify a suture question	69

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

swelling report swelling accurately	69
telephone note complete a telephone note	68
tooth notation confirm the tooth notation system	51
topical anesthetic clarify a topical anesthetic question	41
traceability maintain traceability	15
traceable correction make a traceable correction	51
tracking event interpret a tracking event	60
treatment estimate request a treatment estimate	6
treatment indication explain the treatment indication	33
treatment option compare treatment options	5
treatment plan explain the treatment plan	5
triage route use the triage route	68
trigger ask about a reported trigger	42
try-in arrange a try-in appointment	60
unresolved question track an unresolved question	69
unverified match flag an unverified match	51
urgent concern escalate an urgent concern	68
verbatim record the question verbatim	68
verified replacement obtain a verified replacement	15
wedge clarify the requested wedge	24
work authorization confirm work authorization	59
working model identify the working model	59
written direction quote the written direction	68
wrong-patient risk address wrong-patient risk	50

Original cases. Clear boundaries.

These are original fictional teaching conversations, not transcripts of real workers. Names, figures, organizations, and incidents are invented. Sources support terminology and communication aims, not the fictional results or an endorsement of this book.

[1] American Dental Association. Dental Assistant.

[Open the original source](#)

United States career and role terminology. Permitted duties vary by jurisdiction, qualifications, and actual authorization; examples do not grant clinical authority. Checked 10 October 2026.

[2] CDC. Sterilization and Disinfection in Dental Settings.

[Open the original source](#)

Background on processing terminology and the importance of verification records. The fictional readiness dialogue is not an instrument-reprocessing procedure. Checked 10 October 2026.

[3] General Dental Council. Principle Three: Obtain Valid Consent.

[Open the original source](#)

UK professional context for ongoing patient communication and consent. Original language scenarios do not substitute for the applicable consent process. Checked 10 October 2026.

[4] General Dental Council. Principle Four: Maintain and Protect Patients Information.

[Open the original source](#)

UK professional context for clear, accurate, confidential records and traceable amendments. Follow actual local requirements for records and disclosure. Checked 10 October 2026.

Phil Stilwell

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